History of Mental Health Nursing

- **Florence Nightingale (1820–1910)** has been called the founder of nursing.
- Florence worked hard to educate herself in the art and science of nursing.
- Her mission to help the British soldiers in the Crimean War earned her respect around the world as a nurse and administrator.



History of Mental Health Nursing

- **Dorothea Dix (1802–1887)** was a schoolteacher, not a nurse.
- She believed that people did not need to live in suffering and that society at large had a responsibility to aid those less fortunate.
- Her primary focus was the care of prisoners and the mentally ill.
- She lobbied in the United States and Canada for the improvement of standards of care for the mentally ill and even suggested that the governments take an active role in providing help with finances, food, shelter, and other areas of need.
- Because of the efforts of Dorothea Dix, 32 states developed asylums or "psychiatric hospitals" to care for the mentally ill.

History of Mental Health Nursing

- **Linda Richards (1841–1930)** was pushing to upgrade nursing education.
- She was the first American-trained nurse, and in 1882 she opened the Boston City Hospital Training School for Nurses to teach the specialty of caring for the mentally ill.
- Linda Richards was among the first nurses to teach and work seriously with planning and developing nursing care for patients.

History of Mental Health Nursing

- **Hildegard Peplau (1909–1999)** was a nurse ahead of her time.
- She believed that nursing is multifaceted and that the nurse must educate and promote wellness as well as deliver care to the ill.
- In her book, Interpersonal Relations in Nursing (1952), Dr. Peplau brought together some interpersonal theories from psychiatry and melded them with theories of nursing and communication.
- She believed that nurses work in society—not merely in a hospital or clinic—and that they need to use every opportunity to educate the public and follow role models in physical and mental health.

History of Mental Health Nursing

- **Peplau saw the nurse as:**

1. **Resource person**. Provides information.
2. **Counselor**. Helps patients to explore their thoughts and feelings.
3. **Surrogate**. By role-playing or other means helps the patient to explore and identify feelings from the past.
4. **Technical support**. Coordinates professional services.

History of Mental Health Nursing

- She believed in building a collaborative therapeutic relationship between the nurse and the patient. she cites **four stages of this relationship**:

1. **Orientation**. Patient feels a need and a will to seek out help.
2. **Identification**. Expectations and perceptions about the nurse-patient relationship are identified.
3. **Exploration**. Patient will begin to show motivation in the problem-solving process.
4. **Resolution**. Focus is on the patient's developing self-responsibility and showing personal growth.

The Facilities

- **Asylums :** defines as:

1. a place of refuge;
2. protection given to criminals and debtors;
3. an institution for the care of the needy or sick and especially of the insane."

- Patients in asylums were frequently treated less than humanely.

The Facilities

- **Hospitals** As treatment facilities evolved, the term asylum and the connotations associated with it became unpopular.
- In 1753, Pennsylvania Hospital established a facility to treat those with mental disorders.
- The hospital was established by Dr. Thomas Bond and Benjamin Franklin

The Facilities

- **Free-Standing Facilities centers** may be called detoxification (detox) centers, crisis centers, or similar names.
- Many free-standing treatment centers provide care ranging from crisis-only to more traditional 21-day stays. a stay can last up to 120 days.
- This, too, depends largely on the size and needs of the individual community.

The Facilities
The Breakthrough

- **Psychotropic Medications Deinstitutionalization:**
- **Key Concepts**

1. Mental health nursing has a long and rich history. It has evolved from very rudimentary skills before the time of Florence Nightingale to the specialty area of nursing today.
2. Patients with mental illness are treated in many different types of facilities, depending on the diagnosis and the availability of care in the particular community.

The Facilities

3. The 1950s were important years in the mental health field. The first psychotropic medications were developed, making it possible for people to return to their homes and communities (deinstitutionalization). These medications also allowed other treatment forms to be used more effectively.
4. Nurses at all levels of preparation are integral parts of the mental health treatment team. Our observations, documentation, and interpersonal skills make nurses effective tools in patient care.

The Facilities

5. Since 1955, all nursing curriculums are required to provide mental health theory.
6. A series of laws over the past 50 years have provided for money, education, research, and improvements in the care of the mentally ill. Financial difficulties in the insurance and health-care industries contribute to cutbacks in money and services for care and treatment of the mentally ill.

Basics of Communication

- Human beings communicate. Everything people do or say has a message and a meaning. Sometimes, the words and the actions send different meanings to different people. People communicate all the time in everything they do.
- **Communication** is an ongoing process of sending and receiving messages.

Basics of Communication

- **Communication Theory**
- Sender, Receiver, and Interpretation of Message
- One of the challenging parts of communicating with others is that the process requires three parts: a sender, a message, and a receiver.
- That means the sender is only partially responsible for the communication.
- Sender Sends Message to Receiver

Types of Communication

- **Verbal and Written Communication:** Verbal communication is the process of exchanging information by the spoken or written word.
- It is, therefore, the subjective part of the process.
- The expertise a nurse develops in the areas of written and verbal communication is largely responsible for the credibility of that nurse.

Types of Communication

- **Nonverbal Communication:** Nonverbal communication is more subtle, but it is the greatest influence on communication.
- It consists of people's actions, tone of voice, the way they use their body, and their facial expressions.
- It is more subjective since nonverbal communication can be interpreted many different ways by the receiver

Aggressive and Assertive Communication

- **Aggressive Communication** is communication that is not self-responsible. Aggressive statements most often begin with the word "you." Aggressive communication, like aggressive behavior, is meant to harm another person.
- It is a form of the defense mechanism projection, or blaming, and it attempts to put the responsibility for the interaction on the other person.
- Aggressive communication is also highly subjective as demonstrated in nonverbal communication.

Aggressive and Assertive Communication

- **Assertive Communication** on the other hand, is self-responsible. Assertive statements begin with the word "I." They deal with thoughts and feelings, and they deal with honesty

Therapeutic Communication

- In therapeutic communication, the nurse understands that in order to acquire certain desired information from the patient, unique techniques of communication will have to be instituted.
- They will be individualized to the patient as well as to the mental health disorder.
- Therapeutic communication is a language of its own. It requires testing new methods of communicating and new ways of listening.

Therapeutic Communication

- Therapeutic communication is purposeful: Nurses are trying to determine the patient's needs. Sometimes, for various reasons, the patient is not comfortable sharing his or her needs and concerns.
- At those times, it is up to the nurse to try to uncover the problem by using two tools: techniques of therapeutic communication and "active" or "purposeful" listening (or "listening between the lines").

■ ■ ■ Key Concepts

1. Humans cannot not communicate. Interpersonal communication is a complex process.
2. Therapeutic or helping communication is a language that is learned and shared by nurses. It is a purposeful skill that requires practice.
3. People communicate verbally and non verbally. Nonverbal communication sends a stronger message than verbal communication.

■ ■ ■ Key Concepts

4. Communication can be assertive or aggressive. Assertive statements are the more helpful of the two; they start with the word "I." Aggressive statements are designed to place responsibility on another person. They start with the word "you."
5. Nurses need to be aware of what blocks therapeutic communication.
6. Nurses need to be aware what techniques to use to encourage effective, helping communication with patients.
7. Special techniques are used when communicating with populations who have special communication needs.

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History of Mental Health Nursing

Abraham Maslow (1908–1970)

Abraham Maslow is one of a group of theorists described as person-centered, patient-centered, or humanist. Person-centered theories involve observing and treating the whole person.

Nursing is highly centered in the person-centered and behaviorist theories. One of the main ideals embraced by the nursing profession is Maslow's Hierarchy of Needs. This hierarchy, or orderly progression of development, takes in the physical components of personality development as well as the emotional components. Self-esteem is a tenet of humanistic psychology. Maslow's Hierarchy of Needs has five levels. Maslow said that one must pass through these stages in order and that it is not possible for a person to move up to the next level until the previous level has been mastered.

Abraham Maslow (1908–1970)

Many times this hierarchy is depicted as a large triangle or a staircase to help visualize the progression from the "basic" needs to the "higher" needs of people. The steps are as follows:

1. **Physiological needs**
2. **Safety and security**
3. **Love and belonging**
4. **Self-esteem**
5. **Self-actualization**

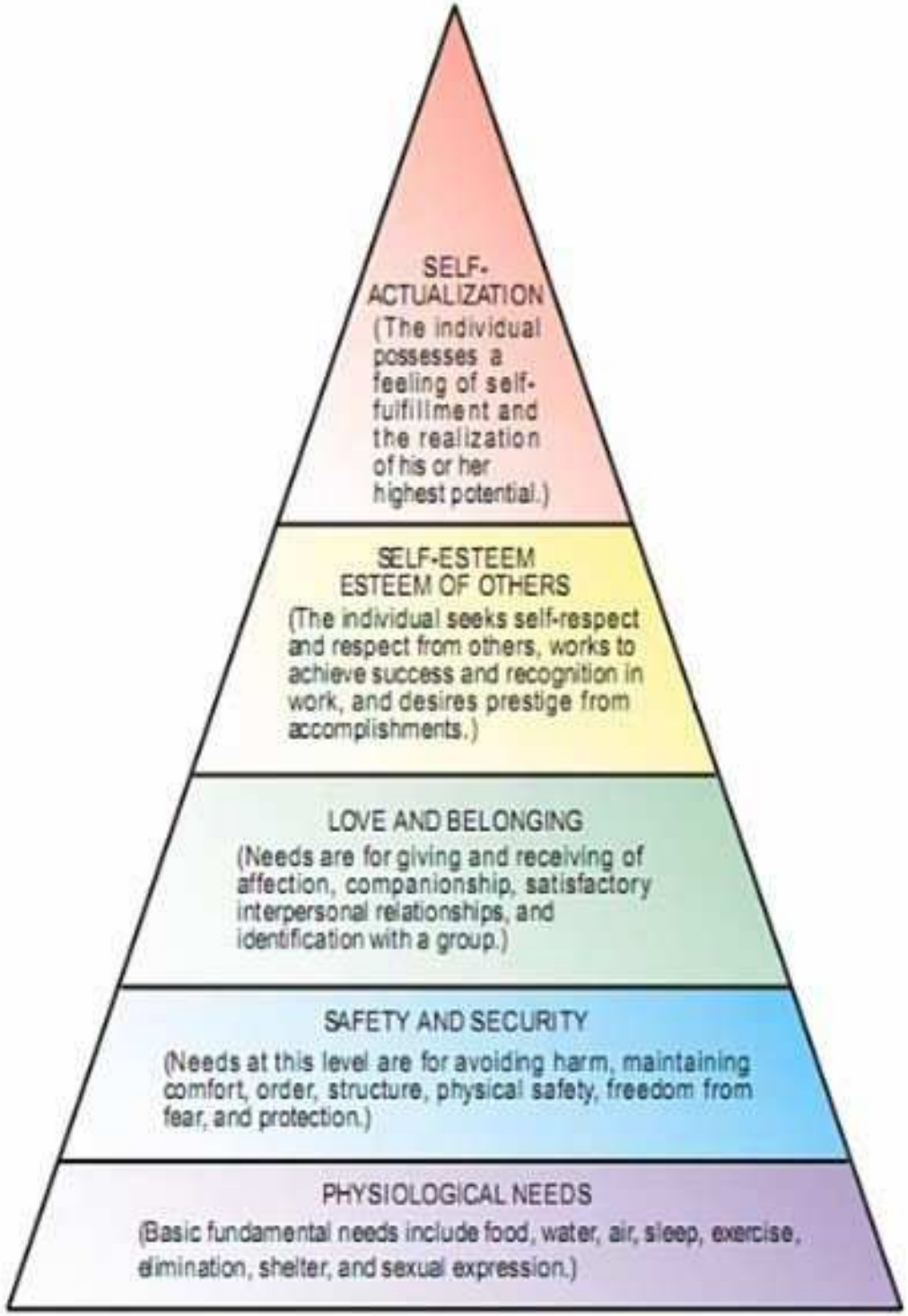


Figure 4-9 Maslow's Hierarchy of Needs. *(From Townsend (2011): Essentials of Psychiatric Mental Health Nursing, 5th ed. Philadelphia: F.A. Davis Company, with permission.)*

Carl Rogers (1902–1987)

Carl Rogers was also a person centered or humanistic psychologist. Although he believed that all people need to be "prized and loved," his theory is a bit different from Maslow's. The phrase associated with Carl Rogers is "unconditional positive regard." Rogers believed that each individual may have different ideas about life and the world he or she lives in. He did not think it was appropriate to put a value on another person's perception of the world, so he said that every person deserved to be treated with respect and "unconditional positive regard" just by virtue of being a human being.

He also differed from Maslow in the area of self-actualization. Rogers believed that self-actualization is the basic motivator for people and that all people have a built-in desire to achieve their capabilities.

Nursing practice is based very strongly in Rogers' theory. Rogers'

- **Eight Steps**

Empathy **"Walk in another's shoes."**

Respect **Care for patient as a *person,* not just a patient.**

Genuineness **Helper is a sincere/authentic role model.**

Concreteness **Identify patient's feelings by careful listening and stereotyping.**

Confrontation **Discuss discrepancies in behavior.**

Self-Disclosure **Share self, as is appropriate to situation.**

Immediacy of Relationships **Helper selectively shares own feelings.**

Self-Exploration **The more we explore ourselves, the greater/better the coping/adapting.**

Five Stages of Grief/Death and Dying by Dr. Elisabeth Kübler-Ross

Denial Key Words "Not me!"

Expected Behaviors Refuses to believe that death is coming; states "That doctor doesn't know what he/she is talking about!"

Anger Key Words "Why me?"

Expected Behaviors Expresses envy, resentment, and frustration with younger people and/or those who are not dying

Bargaining Key Words "If I could have one more chance . . ."

Expected Behaviors May become very religious or "good" in an effort to gain another chance at life or more time to live

Grief/Depression Key Words Realizes that "bargaining" is not working and death is approaching

Expected Behaviors Becomes depressed, weepy; may "give up," quit taking medications, quit eating, and so forth

Acceptance Key Words "OK . . . but I don't have to like it!

Expected Behaviors Enters a state of expectation; may begin to call family members near; needs to complete "unfinished business"; prepares spiritually to die

Appearance **Objective and subjective observations such as dress, hygiene, posture; and about the patient's actions and reactions to health-care personnel.**

Clean, hair combed; clothing intact and appropriate to weather or situation. Teeth in good repair. Posture erect. Cooperates with health-care personnel.

Displays either unusual apathy or concern about appearance.

Behavior **Objective**

Level of Awareness **Cooperates with health-care personnel. Displays uncooperative, hostile, or suspicious-type behaviors toward health-care personnel.**

Subjective and objective assessment of the patient's degree of alertness (wakefulness). Awareness is measured on a continuum that ranges from unconsciousness to mania. "Normal alertness" is the desired behavior.

There is usually a standard guideline for helping with this assessment, but subjective observations can be documented as well, if the

patient cannot stay awake for even short intervals or is overly active and has difficulty staying in one place for any period of time.

Outcome is not within normal limits if the patient is difficult to arouse and keep awake or finds it difficult to feel calm.

Orientation **The degree of patient's knowledge of self.**

Orientation measures the person's ability to know who he or she is, where he or she is, and the day and time, usually within 1 or 2 days of the actual day and time. Measurement techniques are accomplished by asking the patient, "What is your name?" "Where are you right now?" and "Tell me what the day and date are." Asking "Who is the president of the United States?" is used here as well. Nurses frequently document this as "oriented ×3," but it is best to also write down the objective data on which this routine answer is based.

Abnormal results of orientation are the i patient's inability to correctly answer questions pertaining to the patient or to commonly known social information

Thinking/Content of Thought **Subjective assessment of what the patient is thinking and the process the patient uses in thinking.**

Formal testing may be undertaken by the psychologist or psychiatrist to determine the patient's general thought content and pattern.

Nurses may contribute to the assessment of thought by documenting statements the patient makes regarding daily cares and routines.

Behaviors including flight of ideas, loose associations, phobias, delusions, and obsessions may become apparent.

Memory **Subjective assessment of the mind's ability to recall previously known recent and remote (long-term) information. Recent memory: Recall of events that are immediately past or up to within 2 weeks before the assessment. One measurement technique is to verbally list five items. After 1 minute, patient should be able to recall 4–5 of those items. Continue with assessment and at 5 minutes, patient should be able to recall 3–4 of the items. Remote memory: Recall of events of the past beyond 2 weeks prior to assessment. Patients are often asked questions pertaining to where they were born, where they went to grade school, and so on. Inability to accurately perform recent or remote recall exercises within parameters; may indicate symptoms of delirium or dementia.**

Developmental Psychology Throughout the Life Span

Developmental Psychology Throughout the Life Span

The study of developmental psychology encompasses the study of human growth and development, which is a specialty subdivision of psychology.

Many scientific studies have been performed in the specific disciplines; it has yet to be proven that any one is true for everyone in every instance. Each person is unique. Individuals are subjected to different factors such as genetics and environment, which may affect development. These are only theories. Each person develops at his or her own pace.

Developmental Theorists:
Sigmund Freud (1856–1939)

The theories of Sigmund Freud are considered controversial in today's world. Sigmund Freud was an Austrian neurologist. He believed, after observing behaviors of children, that the *personality was developed* as early as age 5 years and fully developed by age 12 years. He said that the personality must develop in a certain way and at strictly defined ages and that failure to progress in this manner would certainly lead to dysfunction. One of Freud's main tenets, or beliefs, is that behaviors resulting from ineffective personality development are unconscious.

He believed that ineffective personality development was in some way related to the relationship of the *child to the parent* and that it was related to what he called psychosexual development. Freud was the first to also offer a reasonably organized method of treatment. Table 4-1 shows Freud's psychosexual or psychoanalytic stages of development. Included in the table are some of the expected behaviors In addition to his five psychosexual stages of development, Sigmund Freud had a model for the components of personality.

Freud's Stages of Development (Psychoanalytic or Psychosexual Stages)

***Oral* Birth– (18 months)**

Tasks/Characteristics Use mouth and tongue to deal with anxiety (e.g., sucking, feeding)

Examples of Unsuccessful Task Completion Smoking, alcoholism, obesity, nail biting, drug addiction, difficulty trusting

***Anal* (18 months–3 years)**

Tasks/Characteristics Muscle control in bladder, rectum, anus provides sensual pleasure and parent pleasing; toilet training can be a crisis

Examples of Unsuccessful Task Completion Constipation, perfectionism, obsessive compulsive disorder

Phallic **(3–6 years)**

Tasks/Characteristics • Learn sexual identity and awareness of genital area as source of pleasure; conflict ends as child represses urge and identifies with same-sex parent

- ***Electra Complex*****: "Penis envy"—Daughter wants father for herself; discovers boys are different from her**
- ***Oedipus Complex*****: Son wants mother to himself; father is a rival**

Examples of Unsuccessful Task Completion

Homosexuality, transsexuality, sexual identity problems in general, difficulty accepting authority

Latency *(*6–12 years)

Tasks/Characteristics Quiet stage in sexual development; learns to socialize

Examples of Unsuccessful Task Completion

Inability to conceptualize; lack of motivation in school or job

Genital *(*12 years–adulthood)

Tasks/Characteristics Sexual maturity and satisfactory relationships with the opposite sex

Examples of Unsuccessful Task Completion

Frigidity, impotence, premature ejaculation, serial marriages, unsatisfactory relationships

Sigmund Freud had a model for the components of personality. He said that the personality consists of three parts: the id, the ego, and the superego. Remember that Freud believed that all the components of human behavior are set in the unconscious. The behaviors may appear to be very purposeful and deliberate, but in Freud's theories, they are supposedly responses to situations of which people are not aware. Id is the part of the personality that is concerned with the gratification of self. He sayings "pleasure principle" or "if it feels good, do it!" are attitudes that arose from those who believe that all people have underdeveloped ids.

These individuals promote the idea that people need to allow the id to take care of "me, myself, and I."

Ego, in Freud's world, had a different connotation from the modern-day common use of the word. Ego, as Freud taught, is the balance to id. Ego keeps id under control (in a mentally healthy individual) by responding in an unconscious form of a "now, wait a minute" attitude. For example, perhaps you had an exam that was in a subject you felt fairly confident about, so you chose to study less than you would for other exams. You went partying with friends for the weekend instead. Think about this as id behavior.

As you entered the testing area, a gnawing feeling started to enter your consciousness. You sensed "butterflies" in the pit of your stomach. You saw the first question on the exam and your mind went temporarily blank. That is the ego response. It's telling you there are two sides to every situation. In this scenario, the ego is telling the id, "Hmm. Maybe you aren't quite as confident as you thought you were!" And the id says, "This test was made just for me."

The third part of the personality theory of Sigmund Freud is the superego. The superego could be called the "killjoy" of the personality.

It is the conscience. It is the part of the personality that allows people to determine what is right, wrong, good, and bad. According to Freud, having these values is not a matter of choice or of learning.

A person who is well-adjusted, or mentally healthy, has all three components of the personality, according to Freud. Freud would expect anyone in whom any of the components is absent or out of balance to display maladaptive behaviors. Defense mechanisms have been associated strongly with Freud's theories.

Erik Erikson (1902–1994)

Erik Erikson was a psychoanalyst and a follower of Freud. Erikson took Freud's main concepts and expanded them to include nonphysical criteria. Erikson understood that people are individuals and that no matter how young the person, everyone is different. Erikson's observations indicated a variable that was different from the psychosexual and age-specific theory offered by Freud. That variable is called an emotional component.

Erikson's Eight Stages of Development.

Table 4-2 Erikson's Eight Stages of Development

Stage	Approximate Ages	Developmental Tasks	Examples	Examples of Unsuccessful Task Completion
Sensory	Birth–18 months	Trust vs. mistrust	Nurturing people build trust in the newborn.	Suspiciousness, trouble with personal relationships
Muscular	1–3 years	Autonomy vs. shame and doubt	"No!"—Toddler learns environment can be manipulated.	Low self-esteem, dependency (on substances or people)
Locomotor	3–6 years	Initiative vs. guilt	Child learns assertiveness can manipulate environment—disapproval leads to guilt in the toddler.	Passive personality, strong feelings of guilt
Latency	6–12 years	Industry vs. inferiority	Creativity or shyness develops.	Unmotivated, unreliable

Adolescence	12–20 years	Identity vs. role confusion	Individual integrates life experiences or becomes confused.	Rebellion, substance abuse, difficulty keeping personal relationships; may regress to child-play behaviors
Young Adult	18–25 years	Intimacy vs. isolation	Main concern is developing intimate relationship with another.	Emotional immaturity; may deny need for personal relationships
Adulthood	21–45 years	Generativity vs. stagnation	Focus is on establishing family and guiding the next generation.	Inability to show concern for anyone but self
Maturity	45 years–death	Integrity vs. despair	Individual accepts own life as fulfilling; if not, he or she becomes fearful of death.	Has difficulty dealing with issues of aging and death; may have feelings of hopelessness

Ivan Pavlov (1849–1936) and B. F. Skinner (1904–1990)

Pavlov and Skinner worked on "conditioning," or manipulating, behaviors. They are called behavioral theorists because they believed that working with different behaviors and different stimuli could obtain different responses. Behavior modification is a direct result of their work. Pavlov worked on *involuntary responses*.

His well-known study was carried out with dogs, steaks, and a bell. When the dogs saw a choice piece of meat, they salivated in preparation for eating it. Pavlov incorporated the ringing of a bell when the meat was presented so that, in time, the researcher rang the bell and the dogs' association of meat with the sound of the bell stimulated the salivation response.

This was a great breakthrough in the study of causes of behavior and ways in which behavior can be manipulated. B. F. Skinner worked on operant conditioning, which is based on voluntary responses. Operant conditioning, means taking a behavior and operating on it by changing the variables or conditions surrounding the behavior.

Skinner is known for the "Skinner boxes" in which he kept the animals he studied. these so-called boxes were cages big enough for the animal to move around in and contained an apparatus for the animal to operate voluntarily in response to different stimuli. There are three main parts to Skinner's theory: response, stimulus, and re inforcer. Skinner's theory led to the development of behavior modification. It is possible to "modify" or change any behavior by using appropriate stimuli and reinforcers to obtain the desired behavior. Both positive and negative behaviors can be changed.

Today, it is generally believed that positive reinforcing is the most effective way of changing a behavior. Pointing out the positive qualities in a person or patient or focusing on the abilities (**positive) rather than the disabilities (negative) seems to yield the best results.**

Operant Conditioning: B. F. Skinner Skinner's Theory

Explanation

Response **Any movement or observable behavior that is to be studied. The response is measured for frequency, duration, and intensity (e.g., chicken rings bell in cage).**

Stimulus **The event that immediately precedes or follows the operant behavior. The object is to find the stimulus that gets the chicken to ring the bell (e.g., food, noise, boredom).**

Reinforcer **A variable that will cause the operant behavior to repeat predictably or increase in frequency. Sometimes this is called a "reward."**

The reinforce has to be meaningful to the person whose behavior is being "operated" on (e.g., chicken pecks bell and food drops into tray; when chicken wants food, it knows that pecking the bell will produce food).



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MENTAL HEALTH NURSING

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■ Somatic Symptom and Related Disorders

Somatic Symptom Disorder (SSD)

Somatic refers to the body. SSD is characterized by somatic symptoms that are either very distressing or result in significant disruption of functioning, as well as excessive and disproportionate thoughts, feelings, and behaviors regarding those symptoms To be diagnosed with SSD, the individual must be persistently symptomatic (typically at least for 6 months). It is a category of disorders in DSM-5 as well as a specific diagnosis.

A prominent feature is excessive focus on one's physical symptoms, that interferes with one's daily functioning. These symptoms may or may not be associated with an actual medical condition. For example, a patient with a small sore seeks multiple opinions out of fear of skin cancer. Despite negative biopsies, the patient keeps checking it, talking about it, and seeking other possible providers who might offer other tests despite great financial burden.

In SSD high levels of worry about one's health becomes the central focus in the person's life even to the point of becoming the person's identity. These patients are high users of the health-care system, often seeking out different specialists and testing. So nurses will encounter these patients more often in nonpsychiatric settings. There is a definite anxiety component to this disorder as the individual worries excessively. Although distress is normal with a new symptom, exaggerated responses before a diagnosis would be a factor in considering SSD diagnosis. In the past the term hypochondriasis might be used to describe someone with SSD. A patient who focuses extensively on physical symptoms is sometimes referred as suffering from somatization or somatizing.

Etiology of Somatic Symptom Disorder

Etiology of Somatic Symptom Disorder

Biologically, research has been conducted concerning the possibility of a genetic or biological predisposition to somatic difficulties. For example some individuals may have increased sensitivity to pain. Early childhood traumatic experiences are also associated with SSD. Psychological theories include physical symptoms rooted in unconscious mechanisms that develop to deny, repress, or displace anxiety.

Somatic Symptom Related Disorders In addition to 1.SSD, several related disorders are covered in this text including 2.conversion disorder, 3.illness anxiety disorder, and 4.factitious disorder. In DSM-IV-TR additional diagnoses included dysmorphophobia and somatoform pain disorder.

Conversion Disorder

Conversion Disorder

Conversion reaction, as defined in the defense mechanisms, is converting anxiety into a physical symptom. Conversion disorder is the illness that emerges from overuse of this mechanism. In conversion disorder, there is a loss or decrease in physical functioning that cannot be explained by any known medical disorder or pathophysiological mechanism. Paralysis and blindness are two of the more common examples of this disorder. It is common for the dysfunction to somehow be deeply connected to denial and to a prior negatively perceived experience (e.g., someone who loses the sense of vision after watching a pornographic movie).

Age of onset is usually adolescence and young adulthood, but it can occur later in life as well. Conversion disorder is also referred to as Functional Neurological Symptom Disorder. The rationale is that persons diagnosed with Functional Neurological Symptom Disorder will likely be seen by a neurologist. The symptoms, although not supportive of organic disease, are very real to the patient. The nurse should not convey to a patient that he or she thinks the patient is "faking" the illness; it is real to the patient. The patient is truly experiencing the symptoms

Even though the patient is concerned enough about the symptoms to consult a physician, he or she may give the impression of really not caring about the problem. "La belle in-difference" is the clinical term used to describe this condition. The belief about this disorder is that the symptom, e.g., the paralysis or blindness, is allowing the patient to avoid a situation that is unacceptable to him or her. This unacceptable situation is the source of extreme anxiety, which is converted into the dysfunction. The dysfunction, then, is relieving the anxiety.

This is called primary gain and is believed to be the function of the paralysis or blindness. Secondary gain is the extra benefits one may acquire as a result of staying ill. Secondary gain includes extra emotional support such as sympathy and love or financial benefits. Much of this is occurring at the unconscious level.

Malingering is a situation for achieving personal gain that differs from the others mentioned. Malingering is a conscious effort to avoid unpleasant situations.

The patient "fakes" or pretends to have the symptoms.

Illness Anxiety Disorder

In this disorder somatic symptoms are not present or if present, • are only mild in intensity. The person's distress is not from the physical complaint itself but rather from his/her anxiety about the meaning, significance, or cause of the complaint. Historically these patients are often referred to popularly as hypochondriacs. These people are sometimes referred to as "professional patients." Hypochondriasis has been a recognized, official diagnosis according to DSM-IV-TR. In DSM-5 hypochondriasis was changed to Illness Anxiety Disorder.

A major difference between illness anxiety disorder and conversion disorder is that the person with conversion disorder focuses on the symptoms of the illness, while the person with illness anxiety disorder is afraid he or she will get a serious disease

Factitious Disorder

Falsification of medical or psychological signs and symptoms in oneself or others is called a factitious disorder. The diagnosis requires demonstrating that the individual is taking secretive actions to misrepresent, simulate or cause signs or symptoms of illness or injury in the absence of obvious external rewards. When the individual falsifies information for another as in a child or pet, the diagnosis is factitious disorder imposed on another or by proxy

Medical Treatment of Patients With Somatic Symptom and Related Disorders

Patients with these disorders are usually admitted to a medical unit rather than a psychiatric unit. Treatment focuses on the symptoms, which more than likely are medical in nature.

Treatment is, of course, individualized for each patient. Once a somatic disorder is diagnosed, the ongoing involvement of a psychiatrist is helpful to give insight to managing this patient. Some approaches that may be used to treat these disorders include individual and group psychotherapy, hypnosis and relaxation techniques

. It is beneficial for the therapist to help the patient express the underlying cause of the anxiety. Hypnosis can be very effective in making this determination. Behavior modification can be effective if the patient is prone to secondary gains from the somatic symptoms. Methods of stress management are also taught as the person learns new ways to handle anxiety. Patients may resist accepting that their problem has a strong psychological or emotional component and therefore cannot understand how a paralyzed limb or pain has anything to do with anxiety. People who have a somatic symptom disorder may feel insulted, become resistive to treatment, and search for other ways to explain the physical problem.

Commonly Used Medications for Somatic Symptom and Related Disorders

Medications are ordered judiciously for these disorders. When a medication is used, it is generally an antidepressant or an antianxiety agent.

Doxepin Bupropion (Wellbutrin) Amitriptyline (Elavil)
Paroxetine (Paxil) (Sinequan) Fluoxetine (Prozac)
Sertraline (Zoloft)

Trazodone (Oleptro)

Nursing Care of the Patient With Somatic Symptom and Related Disorders

Common nursing diagnoses include the following:

- Anxiety
- Coping, ineffective
- Sensory perception, disturbed
- Thought processes, disturbed

Nursing Care for Patients With Somatic Symptom and Related Disorders

Somatic Symptom Disorder

Symptoms

- High level of anxiety about health
- Excessive time and energy devoted to symptoms
- May or may not have an organic disorder

Nursing Interventions

- Listen to patient's concerns but then focus on other issues
- Promote trust
- Encourage patient to express self about other issues than the symptoms

Conversion Disorder

Symptoms

- Loss or decrease in physical functioning that seems to have a neurological connection (paralysis, blindness)
- Indifference to the loss of function
- Primary and secondary gain

Nursing Interventions

- Use therapeutic communication skills.
- Encourage therapy (occupational therapy, physical therapy, etc.).
- Provide emotional support.
- Respond to the patient's symptoms as real

Illness Anxiety Disorder

Symptoms

- "Professional patient"
- Intense fear of becoming seriously ill
- Preoccupation with the idea of being seriously ill and not being helped—may be concerned about not being taken seriously or evaluated properly

Nursing Interventions

- Do not reinforce the symptom.
- Be nonjudgmental.
- Continue to focus on trusting relationship

■ ■ ■ Key Concepts

1. Anxiety disorders have many common characteristics. Psychoanalytic theories propose that it is important to find the underlying cause of the anxiety. Biological theories postulate that the causes are not the primary concern, but rather the physical reasons may result in the anxiety.

2. Medications and therapies should be individualized for the patient.

3. Trust and communication techniques are important tools for the nurse caring for a patient with an anxiety disorder. Maintaining a calm milieu is also essential.

4. Somatic symptom disorder is characterized by somatic symptoms that are either very distressing or result in significant disruption of functioning, as well as excessive and disproportionate thoughts, feelings, and behaviors regarding those symptoms. The symptoms may or may not have an organic cause.

5. Treatment and nursing care for patients with somatoform disorders may be difficul and long term, as these are chronic disorders. Patients may use the defense mechanisms of denial and conversion reaction.

Post-Traumatic Stress Disorder

(PTSD) is developed in response to an unexpected emotional or physical trauma that could not be controlled. A victim of PTSD will probably have reoccurring, intrusive, disturbing memories of the incident that may last over a period of time. It is viewed less as an anxiety problem and more related to the physical and emotional responses to a trauma. One reason for this change is the increasing recognition that people suffering from PTSD often are more troubled with pervasive sadness, aggressive behaviors, and dissociative symptoms such as flashbacks than anxiety symptoms. People who have fought in wars, who have been raped, or who have survived violent storms or other actual or threatened traumatic events are examples of those who are susceptible to suffering from this disorder. Police, fire, and rescue personnel are at risk for PTSD when they see victims of violence and destruction whom they cannot help.

- PTSD symptoms may appear immediately or be repressed until years later. Symptoms of PTSD can include:
- • "Flashbacks," in which the person may relive and act out the traumatic event
- • Social withdrawal
- • Nightmares • Insomnia
- • Feelings of low self-esteem as a result of the event
- • Changes in the relationship with a significant other and difficulty forming new relationships
- • Irritability and outbursts of anger toward another person or situation, apparently for no obvious reason
- • Depression
- • Distress when thinking about the event
- • Making efforts to avoid reminders of the event

Trust and communication and listening skills are very important tools for nurses who have patients with PTSD. Encouraging expression of thoughts and feelings surrounding the experience and the survivor guilt is an important first step in the patient's ability to identify the source of the problem and begin the process of healing. It is important to validate the patient's feelings regarding the situation. Honesty and genuineness in communicating with these patients will help to build a Family members and significant others such as spouses can suffer from the effects of PTSD as well. Often, these people experience the same trauma, even though they were not present for the original event. The term "vicarious trauma" may apply in this situation working rapport..

Family members and significant others such as spouses can suffer from the effects of PTSD as well. Often, these people experience the same trauma, even though they were not present for the original event. The term "vicarious trauma" may apply in this situation.

Medical Treatment of People With Anxiety and Anxiety-Related Disorders

Treatment is individualized to the patient and may include one or more of the following:

psychopharmacology, individual psychotherapy, group therapy, systematic desensitization, hypnosis, imagery, relaxation exercises, and biofeedback.

Medications are being used effectively to control chronic anxiety. The most common are anti-anxiety medications, which include benzodiazepines.

The antidepressant class of selective serotonin reuptake inhibitors (SSRIs) is being used as primary treatment in many cases of GAD, panic disorders, social phobias, OCD, and PTSD.

. For example, the FDA has approved fluoxetine, paroxetine, and fluvoxamine for treatment of OCD. For PTSD, paroxetine and sertraline. Panic disorders have been successfully treated with paroxetine, fluoxetine, and sertraline.

Commonly Used Anti-Anxiety Medications

Chlordiazepoxide Buspirone (BuSpar) Alprazolam (Xanax)
Clonidine (Catapres) Clonazepam (Klonopin) (Librium)
Hydroxyzine (Atarax) Lorazepam (Ativan) Diazepam (Valium)
Zolpidem (Ambien) Prazepam (Centrax) Oxazepam(Serax)

General Intervention

1. Maintain a calm milieu.
2. Maintain open communication.
3. Observe for signs of suicidal thoughts.
4. Document any changes in behavior.
5. Encourage activities

Generalized Anxiety Disorder

Symptoms

Muscle aches, shakes, palpitations, dry mouth, nausea, chills, vomiting, hot flashes, polyuria, difficulty swallowing, feeling of dread

Nursing Actions

- Provide calm milieu
- Open communication done calmly and clearly
- Focus on brief messages
- Teach early signs of escalating anxiety
- Suicide precautions if the person indicates any self-destructive thoughts
- Document behavior changes
- Encourage activities
- Promote deep breathing and other relaxation methods
- Reassurance

Panic Disorder

Symptoms

Fear, dissociation, nausea, diaphoresis, chest pain, increased pulse, shaking, unsteadiness, paralysis

Nursing Actions Same as above

- Stay with patient during attack

Phobia

Symptoms Irrational fear of a particular object or situation

Nursing Actions Same as above

- Focus on nonthreatening topics
- Reassure about patient's safety

Obsessive-Compulsive Disorder

Symptoms Repeated thoughts and/or repeated actions
Nursing Actions Same as above

- Allow patient to express the anxiety
- Recognize and accept need for obsessions and compulsions
- Allow time for rituals
- Provide structured schedule and give patient some control
- Explore alternative methods of anxiety reduction

Post-Traumatic Stress Disorder

Post-Traumatic Stress Disorder "Flashbacks," social withdrawal, low self-esteem, relationships that may change or be difficult to form, irritability, anger seemingly for no reason, depression, chemical dependency

Nursing Actions Same as above

- Keep patient oriented to the present
- Encourage patient and significant others to attend groups for patients with PTSD
- Encourage patient to talk about traumatic events if he/she is able

Nursing Care Plan for Patient With Anxiety

Assessment/ Data Collection

Patient is: • Restless • Irritable • Pacing • Hyperventilating • Verbalizing negative thoughts and expecting a calamity

Diagnosis Anxiety

Plan/Goal Demonstrates a sense of increased comfort

Nursing Interventions/ Nursing Actions

• Calm environment • Promote relaxation techniques including deep breaths

• Soothing music • Verbalize reassurance about current situation resolving

Evaluation Patient appears more relaxed and verbalizes more positive outcomes

Chapter 3
Recording and Reporting

Medical Records

Medical records are written collections of information about a person's health, the care provided by health practitioners, and the client's progress

Also known as health records or client records

Uses of Medical Records

Permanent account: Requested during subsequent admissions.

Sharing information: Prevents duplication of care and helps reduce the chance of error or omission.

Quality assurance: To maintain a high level of care

Accreditation: Examined during an accreditation visit.

Reimbursement: Payment for hospital cost guided by records.

Education and research: Records serve as an alternative resource for scientific data.

Legal evidence: Each person who makes entries in the client's medical record is responsible for the information he or she records and can be summoned as a witness to testify concerning

JCAHO •

Joint Commission on Accreditation of Healthcare – Organizations (JCAHO) establishes criteria reflecting high standards for institutional health care

Representatives of JCAHO periodically inspect health care – agencies and determine evidence of quality care

Based on inspection, agencies are accredited –

Components of Medical Records •

s health information'Person –

Care provided by health practitioners –

s progress'The client –

The plan for care –

Medication administration record –

Laboratory and diagnostic reports –

Criteria for Legally Defensible Charting •

.Ensure that the client's name appears on each page –

.Never chart for someone else –

Use the specified color of ink and ballpoint pen, or enter data on a –
.computer

.Date and time each entry as it is made –

.Chart promptly after providing care –

.Make entries in chronologic order –

Identify documentation that is out of chronologic sequence with –
".the words "late entry

.Write or print legibly –

.Use correct grammar and spelling –

.Reflect the plan of care –

.Describe the outcomes of care –

.Record relevant details –

.Use only approved abbreviations –

Never scribble over entries or use correction fluid to obliterate –
.what has been written

Draw a single line through erroneous information so that it remains readable, add the date, initial, and then document the correct information.

Record facts, not subjective interpretations.

Quote the client's verbal comments.

Write "duplicate" or "recopied" on documentation that is not original; include the date, time, initials, and reason for the duplication.

Never imply criticism of another's care.

Document the circumstances for notifying a physician, the specific data reported, and the physician's recommendations.

Identify specific information provided when teaching a client and the evidence that indicates the client has understood the instructions.

Leave no empty spaces between entries and signature.

Sign each entry by name and title.

Types of Client Records

Source-Oriented Records •

- Organized according to source of documented information
  - Contain separate forms for physicians, nurses, dietitians, physical therapists to make written entries about their specific activities in relation to client's care
- This record provides fragmented documentation

Problem-Oriented Records •

- Organized according to client's health problems
  - Four major components: data base, problem list, plan of care, progress notes
- Information compiled and arranged to emphasize goal-directed care; promote recording of pertinent information; facilitate communication among health care professionals

Components of Problem-Oriented Records

TABLE 9-2 Common Components of a Problem-Oriented Record

COMPONENT	DESCRIPTION
Data base	Contains initial health information
Problem list	Consists of a numeric list of the client's health problems
Plan of care	Identifies methods for solving each identified health problem
Progress notes	Describes the client's responses to what has been done and revisions to the initial plan

Methods of Charting

Narrative charting –

SOAP charting –

Focus charting –

PIE charting –

Charting by exception –

Computerized charting –

Narrative charting ❖

Narrative charting (the style of documentation generally used in source-oriented records) –

There is no established format for narrative notations. –

Narrative charting is time-consuming to write and read. –

person writing a narrative entry may omit pertinent documentation or include insignificant information. –

SOAP charting ❖

- SOAP charting (the documentation style more likely to be used in a problem-oriented record)

:Acquired its name from the four essential components –

subjective data ✓

objective data ✓

A = analysis of the data •

P = plan for care •

- Some agencies have expanded the SOAP format to SOAPIE or SOAPIER (I = interventions, E = evaluation, R revision to the plan of care =

- SOAP charting also helps demonstrate interdisciplinary cooperation

TABLE 9-3 SOAPIER Charting Format

LETTER	EXPLANATION	EXAMPLE OF RECORDING
S = Subjective information	Information reported by the client	S—"I don't feel well."
O = Objective information	Observations made by the nurse	O—Temperature 102.4°F
A = Analysis	Problem identification	A—Fever
P = Plan	Proposed treatment	P—Offer extra fluids and monitor body temperature
I = Intervention	Care provided	I—750 mL of fluid intake in 8 hours; temperature assessed every 4 hours
E = Evaluation	Outcome of treatment	E—Temperature reduced to 101°F
R = Revision	Changes in treatment	R—Increase fluid intake to 1,000 mL per shift until temperature is ≤100°F

Focus Charting •

Modified form of SOAP charting, uses the word – focus rather than problem because some believe that the word problem carries negative .connotations

D = data, A =) focus charting follows a DAR model – DAR notations .(2-9action, R = response; Fig. .tend to reflect the steps in the nursing process

6/30/2007	D(ata) –	Bladder distended 2 fingers above pubis.
1015		Has not urinated in 8 hrs. since
		catheter was removed. ——
	A(ction) –	Assisted to toilet. Water turned on at
		faucet. Instructed to press over bladder
		with hands. ——
	R(esponse) –	Voided 525 mL of clear urine. L. Cass, SN

FIGURE 9-2 Example of DAR charting.

Chapter Three

Schizophrenia and Other Related Disorders

In substance/medication- induced psychotic disorder the psychotic symptoms are judged to be a physiological consequence of a drug of abuse, a medication, or toxin exposure and cease after removal of the agent.

In psychotic disorder due to another medical condition, the psychotic symptoms are judged to be a direct physiological consequence of another medical condition.

Neurosis versus Psychosis

Neurosis

is an exaggerated use of defense mechanisms to escape the feelings of a situation or event that the person wants to avoid.

Types: Anxiety, depression, OCD, PTSD, phobias and eating disorders .

Persons with neurosis can understand that they have a problem that hinders their life routine, but they can't resolve it themselves

Psychotic disorders

Persons do not understand that they have a problem.

Delusions and hallucinations. Thought processes, beleifs and emotions are so impaired, lose contact with reality, great personality changes, dramatic mood swings, abnormal emotional responses and lack of orientation.

Psychosis is marked by absolute distortion from reality and need special treatment and rely on antipsychotic medication rather than counselling.

The three main causes of psychosis include:

- Functional – Mental illness such as schizophrenia and bipolar disorder
- Organic – Chemical imbalance, brain tumors, sleep deprivation
- Psychoactive drugs – Hallucinogens, amphetamines and barbiturates

■ Etiology

The cause of schizophrenia is still uncertain. Most likely, no single factor can be implicated in the etiology; rather, the disease probably results from a combination of influences that include biological, psychological, and environmental factors.

1. Biological Influences

- Genetics:

The body of evidence for genetic vulnerability to schizophrenia is growing. Studies show that relatives of individuals with schizophrenia have a much higher probability of developing the disease than does the general population. Whereas the lifetime risk for

developing schizophrenia is about 1 percent in most population studies, the siblings or offspring of an identified client have a 5 to 10 percent risk of developing schizophrenia.

a) Twin Studies

The rate of schizophrenia among monozygotic (identical) twins is four to five times that of dizygotic (fraternal) twins and approximately 50 times that of the general population.

• Biochemical Influences

The oldest and most thoroughly explored biological theory in the explanation of schizophrenia attributes a pathogenic role to abnormal brain biochemistry. Notions of a "chemical disturbance" as an explanation for insanity were suggested by some theorists as early as the mid-19th century.

a) The Dopamine Hypothesis

This theory suggests that schizophrenia may be caused by an excess of dopamine-dependent neuronal activity in the brain. This excess activity may be related to increased production or release of the substance at nerve terminals, increased receptor sensitivity, too many dopamine receptors, or a combination of these mechanisms.

b) Other Biochemical Hypotheses

Various other biochemicals have been implicated in the predisposition to schizophrenia. Abnormalities in the neuronal activity of the neurotransmitters norepinephrine, serotonin, acetylcholine, and gamma-aminobutyric acid and in the neuroregulators, such as prostaglandins and endorphins, have been suggested.

- Physiological Influences

A number of physical factors of possible etiological significance have been identified in the medical literature. However, their specific mechanisms in the implication of schizophrenia are unclear.

a) Viral Infection

Researchers report that epidemiological data indicate a high incidence of schizophrenia after prenatal exposure to influenza. Another study found an association between viral infections of the central nervous system during childhood and adult onset schizophrenia.

b) Anatomical Abnormalities

Structural brain abnormalities have been observed in individuals with schizophrenia. Ventricular enlargement is the most consistent finding; however, sulci enlargement and cerebellar atrophy are also reported.

c) Histological Changes

Cerebral changes in schizophrenia have also been studied at the microscopic level. A “disordering” or disarray of the pyramidal cells in the area of the hippocampus has been suggested.

d) Physical Conditions

Some studies have reported a link between schizophrenia and epilepsy, Huntington's disease, birth trauma, head injury in adulthood, alcohol abuse, cerebral tumor (particularly in the limbic system), cerebrovascular accidents, systemic lupus erythematosus, myxedema, parkinsonism, and Wilson's disease.

2. Psychological Influences

The early theories implicated poor parent-child relationships and dysfunctional family systems as the cause of schizophrenia, but they no longer hold any credibility. Researchers now focus their studies in terms of schizophrenia as a brain disorder.

3. Environmental Influences

Sociocultural Factors may indicated by statistics, that have shown that greater numbers of individuals from the lower socioeconomic classes experience symptoms associated with schizophrenia than do those from the higher socioeconomic groups.

4. Stressful Life Events

It is known that extreme stress can precipitate psychotic episodes. Stress may indeed precipitate symptoms in an individual who possesses a genetic vulnerability to schizophrenia. Stressful life events may be associated with exacerbation of schizophrenic symptoms and increased rates of relapse.

■ Key features that define the psychotic disorde

Positive or hard symptoms:

1. Delusions

Persecutory delusions are most common, referential delusions are also common.

2. Hallucinations

Auditory hallucinations are usually experienced as voices, that are perceived as distinct from the individual's own thoughts, that are the most common in schizophrenia and related disorders. The second most common type is visual hallucinations, while other hallucinations may be rare.

3. Disorganized Thinking and Speech

In disorganized thinking, the individual may demonstrate loose associations or tangentiality. Rarely, speech may be so severely disorganized that it is nearly incomprehensible (incoherence or "word salad").

4. Grossly Disorganized or Abnormal motor Behavior (including Catatonia)

It may manifest in a variety of ways, ranging from childlike "silliness" to unpredictable agitation that leading to difficulties in performing activities of daily living.

Catatonic behavior is a marked decrease in reactivity to the environment. This ranges from resistance to instructions (negativism); to maintaining a rigid, inappropriate or bizarre posture; to a complete lack of verbal

responses (mutism) and motor responses (stupor). It can also include purposeless and excessive motor activity without obvious cause (catatonic excitement). Other features are repeated stereotyped movements, staring, grimacing, mutism, and the echoing of speech.

Negative or Soft Symptoms

Two negative symptoms are particularly prominent in schizophrenia: diminished emotional expression and avolition.

1. Diminished emotional expression, it may include variety of symptoms as feelings of indifference toward people, activities, and events (Apathy), blunted or flat affect.
2. Avolition is a decrease in motivated self-initiated purposeful activities.

3. Alogia is manifested by diminished speech.

4. Anhedonia is the decreased ability to experience pleasure from positive stimuli or a degradation in the recollection of pleasure previously experienced.

5. Asociality refers to the apparent lack of interest in social interactions.

■ Schizophrenia

Schizophrenia is considered the most common and disabling of the psychotic disorders. This disorder affects all races and is more prevalent in men than women. Schizophrenia occurs twice as often in people who are single or divorced as in those who married or widowed. People with schizophrenia are more likely to be members of lower socioeconomic groups.

The onset of schizophrenia may occur late in adolescence or early in adulthood, usually before the age of 30. Although the disorder has been diagnosed in children, approximately 75% of persons diagnosed as having schizophrenia develop the clinical symptoms between the ages of 16 and 25 years.

Schizophrenia lasts for at least 6 months and includes at least 1 month of active-phase symptoms. During the prodromal or residual periods, the signs of the disturbance may be manifested by only negative symptoms or by two or more positive symptoms in an attenuated form (e.g., odd beliefs, unusual perceptual experiences).

Diagnostic Criteria

A. Two (or more) of the following, at least one of these must be (1), (2), or (3):

1. Delusions.
2. Hallucinations.
3. Disorganized speech
4. Grossly disorganized or catatonic behavior.
5. Negative symptoms

C. Decreased level of interpersonal, academic, or occupational functioning

D. The disturbance is Not due to other psychological condition, or to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition

■ Delusional Disorder

Diagnostic Criteria

A. The presence of one (or more) delusions with a duration of 1 month or longer.

B. Functioning is not markedly impaired, and behavior is not obviously bizarre or odd.

C. The disturbance is not attributable to the physiological effects of a substance or another medical condition and is not better explained by another mental disorder.

Specify if:

With bizarre content

Delusions are deemed bizarre if they are clearly implausible, not understandable, and not derived from ordinary life experiences (e.g., an individual's belief that a stranger has removed his or her internal organs and replaced them with someone else's organs without leaving any wounds or scars).

■ Brief Psychotic Disorder

Diagnostic Criteria

A. Presence of delusions, hallucinations or disorganized speech at least, also individual may has grossly disorganized or catatonic behavior.

B. Duration is at least 1 day but less than 1 month.

C. The disturbance is Not due to other psychological condition, or to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition.

■ Schizophreniform Disorder

Characterized by a symptomatic presentation equivalent to that of schizophrenia except for its duration (last at least 1month but less than 6 months) and the absence of a requirement for a decline in functioning.

Chapter Five

Bipolar and Related Disorders



■ Etiology

Biological Theories

Genetics

Twin Studies

Rate for bipolar disorder among monozygotic twins more than in dizygotic twins.

Family Studies.

Biochemical Influences

Biogenic Amines

Symptoms of depression with a deficiency of norepinephrine and dopamine and mania with a functional excess of these amines.

Electrolytes

Physiological Influences

Neuroanatomical Factors

Medication Side Effects

Steroid therapy, amphetamines, antidepressants, and high doses of anticonvulsants and narcotics also have the potential for initiating a manic episode.

Psychosocial Theories

Such as environmental stressors

The Transactional Model

Bipolar disorder most likely results from an interaction between genetic, biological, and psychosocial determinants. Psychosocial and genetic factors can affect biological factors, such as concentrations of a certain neurotransmitter. Biological and psychosocial factors can also affect gene expression, and biological and genetic factors can affect a person's response to psychosocial factors.

■ Bipolar I Disorder

Diagnostic criteria

For a diagnosis of bipolar I disorder, it is necessary to meet the following criteria for a manic episode. *At least one manic episode* may have been preceded by and may be followed by hypo-manic or major depressive episodes.

- **Manic Episode**

A. A distinct period of abnormally and persistently elevated, expansive, or irritable mood and increased goal-directed activity or energy, lasting at *least 1 week* and present most of the day, nearly every day (or any duration if hospitalization is necessary).

B. Three (or more) of the following symptoms (four if the mood is only irritable) are present to a significant degree and represent a noticeable change from usual behavior:

1. Increased self-esteem or grandiosity.

2. Decreased need for sleep (e.g., feels rested after only 3 hours of sleep).

3. Very talkative or pressure to keep talking.

4. Flight of ideas or thoughts are racing.

5. Distractibility.

6. Increase in goal-directed activity (either socially, at work or school, or sexually) or psychomotor agitation.

7. Excessive involvement in activities that have a high potential for painful consequences (e.g., engaging in unrestrained buying sprees, or foolish business investments).

C. It cause marked impairment in social or occupational functioning or to necessitate hospitalization to prevent harm to self or others, or there are psychotic features.

D. The disturbance is Not due to other psychological condition, or to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition.

- **Hypomanic Episode**

Hypomania is a period of abnormally and persistently elevated, expansive, or irritable mood lasting at least *4 consecutive days* and present most of the day, nearly every day and including three or four of the additional symptoms described earlier in manic episode.

The *difference* is that hypomanic episodes do not impair the person's ability to function (in fact, he or she may be quite productive), and there are no psychotic features.

- **Major Depressive Episode**

Lasts at least *2 weeks*, during which the person experiences a depressed mood or loss of pleasure in nearly all activities.

***Four* of the following symptoms are present every day and cause impairment in functioning:**
Changes in appetite or weight, sleep, or psychomotor activity; decreased energy; feelings of worthlessness or guilt; difficulty thinking, concentrating, or making decisions; or recurrent thoughts of death or suicidal ideation, plans, or attempts.

■ Bipolar II Disorder

At least one episode of major depression and at least one hypomanic episode,

It is no longer thought to be a "milder" condition than bipolar I disorder, largely because of the amount of time individuals with this condition spend in depression and because the instability of mood experienced by individuals with bipolar II disorder is typically accompanied by serious impairment in work and social functioning.

Diagnostic criteria:

A. At least one a current or past hypomanic episode and the criteria for at least one a current or past major depressive episode. There has *never* been a manic episode.

B. It causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

■ Cyclothymic disorder

At least 2 years (for children, a full year) of both hypomanic and depressive periods, and that do not meet criteria for hypomanic episode or a major depressive episode.

The hypomanic and depressive periods have been present for at least half the time and the individual has not been without the symptoms for more than 2 months at a time. The symptoms cause significant distress or impairment in social, occupational, or other areas of functioning.

■ Treatment

Psychopharmacology

Treatment for bipolar disorder involves a lifetime regimen of medications: either an antimanic agent called lithium or anticonvulsant medications used as mood stabilizers (see Chapter 15). This is the only psychiatric disorder in which medications can prevent acute cycles of bipolar behavior.

Psychotherapy

Psychotherapy can be useful in the mildly depressive or normal portion of the bipolar cycle. It is not useful during acute manic stages because the person's attention span is brief and he or she can gain little insight during times of accelerated psychomotor activity. Psychotherapy combined with medication can reduce the risk for suicide and injury, provide support to the client and family, and help the client to accept the diagnosis and treatment plan.

Family Interventions

Although pharmacologic treatment is acknowledged as the primary method of stabilizing an acutely ill bipolar client, adjunctive psychotherapy has been recognized as playing an important role in preventing relapses and improving adjustment. Researchers suggest that involving the family in post-episode stabilization of bipolar disorder is important and helps family members:

Family support is also important in helping the client accept the necessity of ongoing medication administration. Family dynamics and attitudes can play a crucial role in the outcome of a client's recovery. Interventions with family members must include education that promotes understanding that at least part of the client's negative behaviors are attributable to an illness that must be managed, as opposed to being willful and deliberate.

Family-focused psychoeducational treatment (FFT) It is an effective method of reducing relapses and increasing medication adherence in bipolar clients. FFT includes sessions that deal with psychoeducation about bipolar disorder (i.e., symptoms, early recognition, etiology, treatment, self-management), communication training, and problem-solving skills training.

Goals of FFT, include:

Improving communication within the family, teaching the family to recognize the early warning signs of a relapse, teaching the family how to respond to these warning signs, and educating them regarding the necessary treatments. This education helps families gain some control over the conflict that occurs in the post-episode phases.

There is evidence to suggest that the addition of psychosocial therapy enhances the effectiveness of psychopharmacological therapy in the maintenance of bipolar disorder in children and adolescents.

Chapter Six

Depressive disorders



■ The common feature of all of these disorders is the presence of sad, empty, or irritable mood, accompanied by somatic and cognitive changes that significantly affect the individual's capacity to function. What differs among them are issues of duration, timing, or presumed etiology.

■ Large number of substances of abuse, some prescribed medications, and several medical conditions can be associated with depression-like phenomena. This fact is recognized in the diagnoses of substance/medication-induced depressive disorder and depressive disorder due to another medical condition.

■ Epidemiology

Major depression is one of the leading causes of disability in the United States.

Depressive Disorder is higher in women than it is in men by about 2 to 1.

The incidence of bipolar disorder is roughly equal, with a ratio of women to men of 1.2 to 1.

The incidence of depression is higher in young women and has a tendency to decrease with age. The opposite has been found in men, with the prevalence of depressive symptoms being lower in younger men and increasing with age.

There is inverse relationship between social class and depressive symptoms.

The highest incidence of depressive symptoms has been indicated in individuals without close interpersonal relationships and in persons who are divorced or separated.

A number of studies have examined seasonal patterns associated with mood disorders. These studies have revealed two prevalent periods of seasonal involvement: one in the spring and one in the fall. This pattern tends to parallel the seasonal pattern for suicide, which shows a large peak in the spring and a smaller one in October.

■ Etiology

1. Biological Theories

a) Genetics or **hereditary factors (increase in monozygotic rather than dizygotic And in first degree relatives)**

b) Biochemical Influences (**a deficiency of the neurotransmitters norepinephrine, serotonin, and dopamine)**

b) Learning Theory **(Seligman theorized that learned helplessness predisposes individuals to depression by imposing a feeling of lack of control over their life situation).**

c) Object Loss Theory **(Depressive illness occurs as a result of having been abandoned by or otherwise separated from a significant other during the first six months of life).**

d) Cognitive Theory **(Depressive affect is seen as cognitive distortions that result in negative, defeated attitudes)**

■ Major Depressive Disorder (MDD)

A diagnosis based on a single episode is possible, although the disorder is a recurrent one in the majority of cases.

Careful consideration is given to the delineation of normal sadness and grief from a major depressive episode. Bereavement may induce great suffering, but it does not typically induce an episode of major depressive disorder. When they do occur together, the depressive symptoms and functional impairment tend to be more severe and the prognosis is worse compared with bereavement that is not accompanied by major depressive disorder.

Diagnostic Criteria for MDD

A. *Five (or more)* of the following symptoms have been present during the same *2-week* period *nearly every day* and represent a change from previous functioning: at least one of the symptoms is either (1) depressed mood or (2) loss of interest or pleasure, (all symptoms indicated by either subjective account from individual or observation by others):

1. Depressed mood.

2. Markedly decreased interest or pleasure in nearly all activities.

3. Significant unintentional weight loss or gain, or decrease or increase in appetite.

4. Insomnia or hypersomnia.

5. Psychomotor agitation or retardation.

6. Fatigue or loss of energy.

B. The symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

C. The episode is not attributable to the physiological effects of a substance or to another medical condition or to another psychiatric disorders.

7. Feelings of worthlessness or excessive or inappropriate guilt.

8. Diminished ability to think or concentrate, or indecisiveness.

9. Recurrent thoughts of death (not just fear of dying), recurrent suicidal ideation without a specific plan, or a suicide attempt or a specific plan for committing suicide.

Specify if major depressive disorder:

With anxious distress, with mixed features, with melancholic features, with atypical features, with mood-congruent psychotic features, with mood-incongruent psychotic features, with catatonia, with peripartum onset, or with seasonal pattern (recurrent episode only)

■ Persistent Depressive Disorder (Dysthymia)

A more chronic form of depression, persistent depressive disorder (dysthymia), which characterized by at least 2 years of depressed mood most of the day, for majority of days with two (or more) of additional, less severe symptoms that do not meet the criteria for a major depressive episode. During the 2-year period of the disturbance, the individual has never been without the symptoms for more than 2 months at a time.

■ Premenstrual Dysphoric Disorder

In the majority of menstrual cycles, at least five symptoms must be present in the final week before the onset of menses, start to *improve* within a few days after the onset of menses, and become *minimal* or absent in the week post menses.

■ Premenstrual Dysphoric Disorder

One (or more) of the following symptoms must be present:

1. Marked affective lability.

2. Marked irritability or anger or increased interpersonal conflicts.

3. Marked depressed mood, feelings of hopelessness, or self-deprecating thoughts.

4. Marked anxiety, tension, and/or feelings of being keyed up or on edge.

■ Premenstrual Dysphoric Disorder

One (or more) of the following symptoms must additionally be present, to reach a total of *five* symptoms when combined with symptoms from criterion above.

1. Decreased interest in usual activities (e.g., work, school, friends, hobbies).

2. Subjective difficulty in concentration.

3. Lethargy, easy fatigability, or marked lack of energy.

4. Marked change in appetite; overeating; or specific food cravings.

■ Premenstrual Dysphoric Disorder

5. Hypersomnia or insomnia.
6. A sense of being overwhelmed or out of control.
7. Physical symptoms such as breast tenderness or swelling, joint or muscle pain, a sensation of "bloating," or weight gain.

The symptoms must have been met for most menstrual cycles that occurred in the preceding year. Criteria of symptoms should be confirmed by prospective daily ratings during at least two symptomatic cycles.س

- **Atypical depression**

* **Peripartum depression**

* **Depressive disorder with seasonal pattern**

■ Treatment

Psychopharmacology

Major categories of antidepressants include cyclic antidepressants, monoamine oxidase inhibitors (MAOIs), selective serotonin reuptake inhibitors (SSRIs), and atypical antidepressants (See chapter 12).

Other Medical Treatments and Psychotherapy

Electroconvulsive Therapy

Psychiatrists may use electroconvulsive therapy (ECT) to treat depression in select groups, such as clients who do not respond to antidepressants or those who experience intolerable side effects at therapeutic doses (particularly true for older adults). In addition, pregnant women can safely have ECT with no harm to the fetus. Clients who are actively suicidal may be given ECT if there is concern for their safety while waiting weeks for the full effects of antidepressant medication.

Psychotherapy

A combination of psychotherapy and medications is considered the most effective treatment for depressive disorders. There is no one specific type of therapy that is better for the treatment of depression.

- ***Interpersonal therapy*** **focuses on difficulties in relationships, such as grief reactions, role disputes, and role transitions. Interpersonal therapy helps the person to find ways to accomplish this developmental task.**

- ***Behavior therapy*** **seeks to increase the frequency of the client's positively reinforcing interactions with the environment and to decrease negative interactions. It also may focus on improving social skills.**

- ***Cognitive therapy*** **focuses on how the person thinks about the self, others, and the future and interprets his or her experiences. This model focuses on the person's distorted thinking, which, in turn, influences feelings, behavior, and functional abilities.**

Chapter Seven

Obsessive-Compulsive and Related Disorders



■ Body dysmorphic disorder

Body dysmorphic disorder is characterized by preoccupation with one or more perceived defects or flaws in physical appearance that are not observable or appear only slight to others,
and by repetitive behaviors (e.g., mirror checking, excessive grooming, skin picking, or reassurance seeking)
or mental acts (e.g., comparing one's appearance with that of other people) in response to the appearance concerns.

The appearance preoccupations are not better explained by concerns with body fat or weight in an individual with an eating disorder.

Muscle dysmorphia:

Is a form of body dysmorphic disorder that is characterized by the belief that one's body build is too small or is insufficiently muscular.

■ Hoarding disorder

Hoarding disorder is characterized by persistent difficulty discarding or parting with possessions, regardless of their actual value, as a result of a strong perceived need to save the items and to distress associated with discarding them.

Hoarding disorder differs from normal collecting. For example, symptoms of hoarding disorder result in the accumulation of a large number of possessions that congest and clutter active living areas to the extent that their intended use is substantially compromised.

The excessive acquisition form of hoarding disorder, which characterizes most individuals with hoarding disorder, consists of excessive collecting, buying, or stealing of items that are not needed or for which there is no available space.

■ Trichotillomania (hair-pulling disorder)

Trichotillomania (hair-pulling disorder) is characterized by recurrent pulling out of one's hair resulting in hair loss, and repeated attempts to decrease or stop hair pulling.

■ Excoriation (skin-picking) disorder

Excoriation (skin-picking) disorder is characterized by recurrent picking of one's skin resulting in skin lesions and repeated attempts to decrease or stop skin picking.

The body-focused repetitive behaviors that characterize Hoarding disorder and Excoriation disorder are:

- Not triggered by obsessions or preoccupations; however, they may be preceded or accompanied by various emotional states, such as feelings of anxiety or boredom.
- They may also be preceded by an increasing sense of tension or may lead to gratification, pleasure, or a sense of relief when the hair is pulled out or the skin is picked.

- **Individuals with these disorders may have varying degrees of conscious awareness of the behavior while engaging in it, with some individuals displaying more focused attention on the behavior (with preceding tension and subsequent relief) and other individuals displaying more automatic behavior (with the behaviors seeming to occur without full awareness).**

■ Obsessive-Compulsive Disorder

Obsessive compulsive disorders are characterized by the presence of obsessions and/or compulsions.

Obsessions:

Are recurrent, persistent, intrusive and unwanted thoughts, urges, or images, and that in most individuals cause marked anxiety or distress. The individual attempts to ignore or suppress such thoughts, urges, or images, or to neutralize them with some other thought or action (i.e., by performing a compulsion).

Compulsions:

Are repetitive behaviors (e.g., hand washing, ordering, checking) or mental acts (e.g., praying, counting, repeating words silently) that the individual feels driven to perform in response to an obsession. The behaviors or mental acts are aimed at preventing or reducing anxiety or distress, or preventing some dreaded event or situation; however, these behaviors or mental acts are not connected in a realistic way with what they are designed to neutralize or prevent, or are clearly excessive.

Diagnostic Criteria

A. Presence of obsessions, compulsions, or both.

B. The obsessions or compulsions are time-consuming (e.g., take more than 1 hour per day) or cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

C. The obsessive-compulsive symptoms are not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition, or another mental disorder.

Many individuals with obsessive-compulsive disorder (OCD) have dysfunctional beliefs. These beliefs can include an inflated sense of responsibility and the tendency to overestimate threat; perfectionism and intolerance of uncertainty; and over-importance of thoughts (e.g., believing that having a forbidden thought is as bad as acting on it) and the need to control thoughts.

Individuals with OCD vary in the degree of insight they have about the accuracy of the beliefs that underlie their obsessive-compulsive symptoms. Many individuals have *good or fair insight* , while some have *poor insight*, and a few (4% or less) have *absent insight/delusional beliefs.* Insight can vary within an individual over the course of the illness. Poorer insight has been linked to worse long-term outcome.

■ Treatment

Optimal treatment for OCD combines **medication** and behavior therapy.

Behavior therapy specifically includes exposure and response prevention:

Exposure involves assisting the client to deliberately confront the situations and stimuli that he or she usually avoids.

Response prevention focuses on delaying or avoiding performance of rituals. The person learns to tolerate the anxiety and to recognize that it will recede without the disastrous imagined consequences.

Other techniques, such as **deep breathing** and **relaxation**, also can assist the person to tolerate and eventually manage the anxiety.

Chapter Eight

Trauma-and Stressor-related disorder



Trauma–and stressor-related disorders include disorders in which exposure to a traumatic or stressful event.

These include

Reactive attachment disorder

Disinhibited social engagement disorder

Posttraumatic stress disorder (PTSD)

Acute stress disorder (ASD)

Adjustment disorders.

■ Etiology

1. Biological Theory

An endogenous opioid peptide response may assist in the maintenance of chronic PTSD. These studies suggest that physiological arousal initiated by re-exposure to trauma-like situations enhances production of endogenous opioid peptides and results in increased feelings of comfort and control. When the stressor terminates, the individual may experience opioid withdrawal, the symptoms of which bear strong resemblance to those of PTSD.

2. Psychosocial Theory

One psychosocial model that has become widely accepted seeks to explain why certain persons exposed to massive trauma develop trauma-related disorders and others do not.

Variables include characteristics that relate to: (1) the traumatic experience; (2) the individual; and (3) the recovery environment.

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3. Cognitive Theory

Most individuals hold positive beliefs of the world as a source of benevolence and joy and of the self as worthy and in control of his or her life. An individual is vulnerable to trauma-related disorders when his or her fundamental beliefs about the self and the world are invalidated by a trauma that cannot be comprehended and a sense of helplessness and hopelessness prevail.

■ Acute Stress Disorder

Acute stress disorder (ASD) is described by the *DSM-5* as a trauma related disorder similar to PTSD. The similarities between the two disorders occur in terms of precipitating traumatic events and symptomatology; however, in ASD, the symptoms are time limited, up to *1 month* following the trauma. By definition, if the symptoms last longer than 1 month, the diagnosis would be PTSD.

■ Adjustment Disorder

An adjustment disorder is characterized by a maladaptive reaction to an identifiable stressor or stressors that results in the development of clinically significant emotional or behavioral symptoms (APA, 2013).

The response occurs within 3 months after onset of the stressor and has persisted for no longer than *6 months* after the stressor or its consequences have ended.

A number of clinical presentations are associated with adjustment disorders.

The following categories, identified by the *DSM*-5 (APA, 2013), are distinguished by the predominant features of the maladaptive response.

■ Posttraumatic Stress Disorder

Diagnostic Criteria

A. *Exposure* to actual or threatened death, serious injury, or sexual violence in one (or more) of the following ways:

- **Directly experiencing the traumatic event.**
- **Witnessing, in person, the event as it occurred to others.**

- Learning that the traumatic event occurred to a close family member or close friend. In cases of actual or threatened death of a family member or friend, the event must have been violent or accidental.
- Experiencing repeated or extreme exposure to aversive details of the traumatic event (e.g., police officers repeatedly exposed to details of child abuse)

B. Presence of one (or more) of the following *intrusion symptoms* associated with the traumatic event, beginning after the traumatic event occurred:

- **Recurrent, involuntary, and intrusive distressing memories of the traumatic event.**
- **Recurrent distressing dreams that related to the traumatic event.**
- **Dissociative reactions (e.g., flashbacks) in which the individual feels or acts as if the traumatic event were recurring.**

- **Intense or prolonged psychological distress at exposure to internal or external cues that symbolize or resemble an aspect of the traumatic event.**
- **Marked physiological reactions to internal or external cues that symbolize or resemble an aspect of the traumatic event.**

C. ***Persistent avoidance*** or efforts to avoid stimuli associated with the traumatic event, beginning after the traumatic event occurred, as evidenced by ***one or both*** of the following:

- Avoidance of distressing memories, thoughts, or feelings about the traumatic event.
- Avoidance of external reminders (people, places, conversations, activities, objects, situations) that arouse distressing memories, thoughts, or feelings about or closely associated with the traumatic event.

D. *Negative alterations in cognitions and mood* **associated with the traumatic event, beginning or worsening after the traumatic event occurred, as evidenced by two (or more) of the following:**

- **Inability to remember an important aspect of the traumatic event (typically due to dissociative amnesia only).**
- **Persistent and exaggerated negative beliefs or expectations about oneself, others, or the world (e.g., "No one can be trusted," 'The world is completely dangerous,").**

- Persistent, distorted cognitions about the cause or consequences of the traumatic event that lead the individual to blame himself/herself or others.
- Persistent negative emotional state (e.g., fear, horror, anger, guilt, or shame).
- Markedly diminished interest or participation in significant activities.
- Feelings of detachment or estrangement from others.
- Persistent inability to experience positive emotions.

E. Marked *alterations in* *arousal and reactivity* associated with the traumatic event, beginning or worsening after the traumatic event occurred, as evidenced by two (or more) of the following:

- **Irritable behavior and angry outbursts (with little or no provocation) typically expressed as verbal or physical aggression toward people or objects.**
- **Reckless or self-destructive behavior.**

- **Hypervigilance.**
- **Exaggerated startle response.**
- **Problems with concentration.**
- **Sleep disturbance (e.g., difficulty falling or staying asleep or restless sleep).**

F. Duration of the disturbance (Criteria B, C, D, and E) is *more than 1 month.*

G. The disturbance causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

H. The disturbance is not attributable to the physiological effects of a substance (e.g., medication, alcohol) or another medical condition. For summary of symptoms, see Table 8.1

Specify whether:

- **With dissociative symptoms:** The individual's symptoms meet the criteria for posttraumatic stress disorder, and in addition, in response to the stressor, the individual experiences persistent or recurrent symptoms of either of the following:

1. ***Depersonalization***

2. *Derealization*

- **With delayed expression:** If the full diagnostic criteria are not met until at least 6 months after the event (although the onset and expression of some symptoms may be immediate).

■ Treatment

PTSD treatment can help the client to regain a sense of control over their life. The primary treatment is psychotherapy, but often included medication. Combining these treatments is necessary.

Psychopharmacology

Several types of medications can help in the improvement of symptoms of PTSD; which include antidepressants, antianxiety, or hypnotics medications.

Psychotherapy (Talk therapy)

- ***Cognitive therapy*** **helps the client to recognize the ways of thinking that are keeping him/her stuck, its often used along with exposure therapy.**

- ***Exposure therapy*** **helps the client to face frightening situation and cope with it effectively.**

- ***Eye movement desensitization and reprocessing*** **(EMDR) combining exposure therapy with a serious of guided eye movements that help to process traumatic memories and change the way of reacting to it.**

Chapter Nine

Somatic Symptom and Related Disorders



Categories of somatic symptom disorders (APA):

Somatic symptom disorder,
Illness anxiety disorder,
Conversion disorder,
Psychological factors affecting other medical conditions,
Factitious Disorder,
Other specified somatic symptoms and related disorders,
and Unspecified somatic symptoms and related disorders.

■ Somatic Symptom Disorder

Is a syndrome of multiple somatic symptoms that cannot be explained medically and are associated with psychosocial distress and long-term seeking of assistance from health-care professionals.

Symptoms may be vague, dramatized, or exaggerated in their presentation, and an excessive amount of time and energy is devoted to worry and concern about the symptoms.

Individuals with somatic symptom disorder are so totally convinced that their symptoms are related to organic pathology that they adamantly reject and are often irritated by any implication that stress or psychosocial factors play any role in their condition.

The disorder is chronic, with symptoms beginning before age 30.

Anxiety and depression are frequently manifested, and suicidal threats and attempts are not uncommon.

■ Illness Anxiety Disorder

Illness anxiety disorder may be defined as an unrealistic or inaccurate interpretation of physical symptoms or sensations, leading to preoccupation and fear of having a serious disease.

The fear becomes disabling and persists despite appropriate reassurance that no organic pathology can be detected.

Symptoms may be minimal or absent, but the individual is highly anxious about and suspicious of the presence of an undiagnosed, serious medical illness.

Individuals with illness anxiety disorder are extremely conscious of bodily sensations and changes and may become convinced that a rapid heart rate indicates they have heart disease or that a small sore is skin cancer.

They are profoundly preoccupied with their bodies and are totally aware of even the slightest change in feeling or sensation.

Their response to these small changes, however, is usually unrealistic and exaggerated.

Some individuals with illness anxiety disorder have a long history of "doctor shopping" and are convinced that they are not receiving the proper care.

Others avoid seeking medical assistance because to do so would increase their anxiety to intolerable levels.

Depression is common, and obsessive-compulsive traits frequently accompany the disorder.

Preoccupation with the fear of serious disease may interfere with social or occupational functioning.

Some individuals are able to function appropriately on the job, however, while limiting their physical complaints to non-work time.

Patients are so apprehensive and fearful that they become alarmed at the slightest intimation of serious illness.

Even reading about a disease or hearing that someone they know has been diagnosed with an illness precipitates alarm on their part.

■ Conversion Disorder (Functional Neurological Symptom Disorder)

Conversion disorder is a loss of or change in body function that cannot be explained by any known medical disorder or pathophysiological mechanism. There is most likely a psychological component involved in the initiation, exacerbation, or perpetuation of the symptom, although it may or may not be obvious or identifiable.

Conversion symptoms affect voluntary motor or sensory functioning suggestive of neurological disease.

Examples include:

Paralysis,
Aphonia (inability to produce voice),
Seizures,
Coordination disturbance,
Difficulty swallowing,
Urinary retention,
Akinesia,

Blindness,
Deafness,
Double vision,
***Anosmia* (inability to perceive smell),**
Loss of pain sensation,
Hallucinations
***Pseudocyesis* (false pregnancy) may represent a strong desire to be pregnant.**

It is likely that multiple causes play a role in the etiology. Most symptoms of conversion disorder resolve within a few weeks.

■ Treatment

Treatment focuses on managing symptoms and improving quality of life. The health care provider must show empathy and sensitivity to the client's physical complaints. A trusting relationship helps to ensure that clients stay with and receive care from one provider instead of "doctor shopping." Optimal treatment combines medication and psychotherapy.

For many clients, depression may accompany or result from somatoform disorders. Thus, **antidepressants** help in some cases. Selective serotonin reuptake inhibitors are used most commonly.

For clients with pain disorder, referral to a chronic pain clinic may be useful. Clients learn methods of **pain management** such as visual imaging and relaxation.

Services such as physical therapy to maintain and build muscle tone help to improve functional abilities.

Providers should avoid prescribing and administering narcotic analgesics to these clients because of the risk for dependence or abuse.

Clients can use nonsteroidal anti-inflammatory agents to help reduce pain.

Involvement in a structured cognitive-behavioral group is beneficial for some people with somatoform disorders, especially somatization disorders.

CBT help in improvement of physical and emotional health, and may alter the erroneous perceptions of threat (of illness) in hypochondriasis.

The overall goals of the group were offering peer support, sharing methods of coping, and perceiving and expressing emotions.

Chapter One

Foundations

■ The Professional – Client Relationship

The professional – client relationship is a working relationship that is established between a professional and a client or substitute decision maker, based on the individual needs of each client for competent, safe, ethical, professional care / services.

■ The Components of A Professional – Client Relationship

- Power
- Trust
- Respect
- Closeness

■ Conditions Essential for a Therapeutic Relationship

Therapeutic relationship is defined as an interaction between two people (usually a caregiver and a care receiver) in which input from both participants contributes to a climate of healing, growth promotion, and/or illness prevention.

Eight conditions are essential for a therapeutic relationship to occur. They include the following:

- Empathy
- Genuineness
- Self-disclosure
- Concreteness and specificity

- Confrontation
- Immediacy of relationship
- Client self-exploration
- Respect

■ Boundaries of A Professional – Client Relationship

A **"boundary"** defines the comfort zone or personal space of an individual. The "***inner boundary***" is the line that separates the client from the professional. The "***outer boundary***" is the point at which the professional becomes distant and loses touch with the client.

Consequences of poor professional boundaries

- Leads to potentially unsafe practice.
- Increases client/worker stress.
- Undermines the therapeutic relationship.
- Can compound clients difficulties.

- Increases well-being of the worker.
- Provides a therapeutic environment underpinned by mutual respect.

How are professional boundaries established?

- Professional codes of conduct
- By law
- Clinical governance
- Code of ethics

Who Negotiates Boundaries?

- Duty of the worker to act in the best interest of the client
- The client may have a life experience where boundaries have been compromised and/or may not be aware of the need for boundaries,
- The worker is ultimately responsible for managing boundary issues

Potential contexts in which boundary violations can occur

- Touching
- Receiving money or gifts from clients
- Loss of respect within the client/worker relationship
- Having dual relationships with clients
- Client confidentiality
- Self disclosure

Factors that impact on professional boundaries

Organizational Factors:

- Roles
- Culture
- Organizational Structure
- Management and supervision
- Appropriate training

The Client Group:

- The capacity of the client
- Impact of transference and counter-transference

Personal Factors:

- Dual relationship with the client
- Resilience/personal vulnerability
- Valency

Chapter One

Foundations

■ Health Care Ethics

There is often a conflict between the healthcare professionals and patients regarding the best choice. The four principles of healthcare ethics provide us a direction to estimate the significance of the ethical decision in clinical setting. It is important to note here that the application of these ethical principles may vary according to the situation. It also depends on the laws governing to the principle and the conditions applied.

Principles of Health Care Ethics

- **Autonomy**

The principle of autonomy applies in terms of rights and obligations. It emphasizes on the rights of patients to take decisions about their medical treatment.

• Beneficence and Non-maleficence

The ethical principle of beneficence requires healthcare professionals to treat their patients in a way that provides maximum benefit to that patient. While on the other hand, the principle of non-maleficence assures the duty of healthcare staff to avoid causing any harm to the patient.

Conflict between Autonomy and Beneficence

The principle of autonomy often comes into conflict with the principle of beneficence. In some situations, intentionally or unintentionally, professionals use paternalistic approach while caring for their patients as they consider that approach as useful for their patient.

- **Justice**

The literal meaning of ***justice*** is equality and fairness. In healthcare industry, justice refers to equitable distribution of resources.

■ Creating Inviting Environments of Care

The **therapeutic milieu** is an environment that is structured and maintained as an ideal, dynamic setting in which to work with clients. This milieu includes safe physical surroundings, all treatment team members, and other clients.

Milieu therapy

refers to socioenvironmental therapy that is structured and maintained as an ideal, dynamic setting in which to work with clients.

Criteria for Establishing a Therapeutic Milieu

- Be purposeful and planned
- Promote interaction and communication among clients and personnel
- Provide a testing ground for new patterns of behavior while clients take responsibility for their actions.

- Encourage participation in group activities
- Provide for client respect and dignity
- Convey an attitude of overall acceptance and optimism.

Chapter Two

Recognizing Values, Attitudes, and Beliefs

■ Introduction

- **Values** relate to our personal principles, morals, and ideals—that is, what we consider to be important.
- **Attitudes** relate to a person's views, which may be evidenced in the way they behave.
- **Beliefs** relate to those things in which an individual has faith—'religious beliefs'

■ Beliefs

* Rokeach (1969) defines **belief** as "any simple proposition, conscious or unconscious, inferred from what a person says or does".

*** Rokeach (1969) proposes three kinds of beliefs**

1. A **descriptive or existential belief**, which coincides with common facts.

2. An **evaluative belief**, which coincides with an individual's evaluative judgment.
3. A **prescriptive or exhortatory belief**.

A **belief system** that includes the "total universe of a person's beliefs about the physical world, social world, and the self" .

■ The values in healthcare

Peace

Positivity is about having the choice and power to change the way we think

Compassion brings humanity to health care. It is the expression of our innate qualities of patience, generosity and kindness

Co-operation is about working together successfully, as individuals and teams.

Valuing the self requires that we recognise our own worth, and in doing so, can better acknowledge the intrinsic worth of others.

Chapter Two

Recognizing Values, Attitudes, and Beliefs

The tools for learning values in healthcare

- **Meditation** this involves participants in being silent and using the time to learn about their mind and their thoughts.
- **Visualisation** involves using the mind to create positive images which can help to address past negative experiences and associated feelings of failure or frustration.

- **Reflection** 'Reflective practice' involves learning from past experience to review professional progress, evaluate concerns and improve clinical practice.
- **Listening** involves participants in deep listening and requires that the listener finds inner peacefulness.

- **Appreciation** it looks at individuals and groups from the perspective of valuing what works best
- **Creativity** encourages the discovery of new solutions.
- **Playfulness** introduces the idea that it is legitimate to experience fun and laughter as part of the learning process

■ Stigma

Stigma refers to attitudes and beliefs that lead people to reject, avoid, or fear those they perceive as being different.

Types of stigma

- **Public stigma**

Refers to the attitudes and beliefs of the general public towards persons with mental health challenges or their family members.

- **Institutional stigma**

Refers to an organization's policies or culture of negative attitudes and beliefs.

Stigma and Perceptions About Mental Illness

It has four components:

- Labeling someone with a condition
- Stereotyping people who have that condition
- Creating a division between a superior "us" group and a devalued "them" group
- Discriminating against someone on the basis of their label Not all labels are negative or result in stigma.

■ Discrimination

While “stigma” is an attitude or belief, “discrimination” is behavioral because of those attitudes or beliefs.

Discrimination occurs when individuals or institutions unjustly deprive others of their rights and life opportunities due to stigma.

How does discrimination occur?

-- Discrimination includes “disparate or different treatment” on the basis of disability.

-- Discrimination also includes a failure to provide a reasonable accommodation to a person with a disability.

Chapter Three

Multicultural Perspectives

■ The concept of culture

Culture is a broad term referring to a set of shared beliefs, values, behavioral norms, and practices that is common to a group of people sharing a common identity and language.

Culture has two distinctive components

Material culture consists of objects that are related to the material aspect of our life such as our dress, food, and household goods.

Non-material culture refers to ideas, ideals, thoughts and belief.

General Characteristics of Culture

- Culture is learned and acquired
- Culture is shared by a group of people
- Culture is cumulative
- Culture changes
- Culture is dynamic

- Culture gives us a range of permissible behaviour patterns
- Culture is diverse
- Culture is ideational

Cultural Associations

Culture (can be associated with): racial or ethnic group, gender, religion, and economic status. Also culture may be associated with nationality, physical capacity or handicap, and affectional or sexual orientation.

Culture can include

- Demographic variables such as age, gender, and place of residence.
- Status variables such as social, educational , and economic background
- Formal and informal affiliations
- Ethnographic variables: such as nationality, ethnicity, language, and religion

Multicultural Counseling:

Multicultural Counseling is any counseling relationship in which the counselor and the client belong to different cultural groups, hold different assumptions about social reality, and subscribe to different worldviews.

Premises of Multicultural Counseling

- All cultures have ways of coping with problems.
- All counseling can be regarded as multicultural if culture is defined broadly to include: Race, ethnicity, nationality, gender, age, social class, sexual orientation, and disability.

- People seek counseling largely because of problems of social-cultural conditions.
- Traditional counseling was developed in the West to cope with psychological distress.
- All cultures have developed ways of dealing with human problems.

The Problem of Cultural Tunnel Vision

Traits exhibited by culturally encapsulated counselors: Defines reality according to one set of cultural assumptions

More traits of culturally encapsulated counselors

- Lack of sensitivity to culture variations among individuals
- Unwilling to challenge ones assumptions of a particular culture
- Fails to consider other viewpoints and makes little attempt to understand the behavior of others
- Is trapped in one way of thinking

Challenges of Reaching Diverse Client Populations

Counselors have Professional Responsibility to:

- Be aware of and deal with the biases, stereotypes, and assumptions that are part of their practice
- Be sensitive to client's values and world view

- Develop treatment strategies that take into account the social, cultural, historical, and environmental influences of culturally different clients

Learning to Deal with Cultural Pluralism

- Multicultural competence is a developmental process that requires continual education and training.

Characteristics of the Culturally Skilled Counselor

I. Deals with practitioner's attitudes and beliefs about race, culture, ethnicity, gender, and sexual orientation.

II. A culturally competent practitioner is knowledgeable and understanding of their own worldview.

III. Deals with skills, intervention techniques, and strategies necessary in serving diverse client groups.

Our Views on Multicultural Training

- Students should become involved in a self-exploratory class to help identify their cultural and ethnic blind spots
- Students should take a course in multicultural issues
- Multicultural perspectives should be integrated throughout a student's curriculum

■ Concept of Subculture

A subculture is a smaller group that exists within a larger culture. Members of a subculture may share commonalities, such as age, gender, race, ethnicity, socioeconomic status religious and spiritual beliefs, sexual orientation, occupation, and even health status.

■More multicultural terminology

Minority group: has come to refer to any category of people who have been discriminated against or subjected to unequal treatment and oppression by society largely because of their group membership.

Multiculturalism: any relationship between and within two or more diverse groups.

Diversity: individual differences such as age, gender, sexual orientation, religion, and physical ability or disability.

Diversity-Sensitive Counseling: concept that includes age, culture disability, education level, ethnicity, gender, language, race, religion, residential location, sexual orientation, and trauma.

■ Achieving culturally competent care

Cultural competence' relates to an ' individual's ability to treat every person with dignity, respect, and fairness, in a way that is sensitively responsive to differences and similarities, and thereby contributes to creating a genuinely inclusive culture.

■ Culture of Palestine

The **Culture of Palestine** is the culture of the Palestinian people, located across Historic Palestine as well as in the Palestinian diaspora. Palestinian culture is influenced by the many diverse cultures and religions which have existed in historic Palestine, from the early Canaanite period onward.

Cultural contributions to the fields of art, literature, music , costume and cuisine express the Palestinian identity despite the geographical separation between the Palestinian territories, Palestinian citizens and the diaspora.

- **Folklore**

Palestinian folklore is the body of expressive culture, including tales, music, dance, legends, oral history, proverbs, jokes, popular beliefs, customs, and comprising the traditions (including oral traditions) of Palestinian culture.

- **Costumes**

Foreign travelers to Palestine in late 19th and early 20th centuries often commented on the rich variety of costumes among the Palestinian people, and particularly among the fellaheen or village women.

Chapter Five

Communication



■ What is the communication?

Communication is the process that people use to exchange information.
By two ways: verbally and nonverbally.

It is the process of understanding and being understood through ideas, facts, thoughts and emotions.

Elements of effective communication:

1. Clarity of thoughts and emotions
Knows what to convey and way

2. Correteness/ Appropriateness
Message clear and correct.

3. Conciseness
give only information that is necessary and important

Elements of effective communication:

4.Conductive environment

5. Feedback

6. Completeness

Three levels of communication:

1. The content
What someone is saying

2. The procedure
The way someone is saying

3. The process
relationship between speaker and listener
and emotion arising during conversation

A congruent message is conveyed when content and process agree.

An incongruent message when the content and process disagree.

Therapeutic communication :

An interpersonal interaction between the health professional and the client during which the health professional focuses on the client's specific needs to promote an effective exchange of information

Chapter Five

Communication



Verbal communication skills

Verbal communication consists of the words a person uses to speak to one or more listeners. Content is the literal words that a person speaks. Context is the environment in which communication occurs and can include the time and the physical, social, emotional, and cultural environments.

Verbal communication skills include the followings:

1. Using Concrete Messages

Use clear, direct words, easy to understood by the client and need no interpretation; the speaker uses nouns instead of pronouns. Avoid abstract messages.

2. Interpreting Signals or Cues

Watches and listens carefully for cues.

3. Using Therapeutic Communication Technique

Therapeutic communication technique	Meaning
Accepting	**indicating reception**
Broad openings	**allowing the client to take the initiative in introducing the topic**
Encouraging comparison	**asking that similarities and differences be noted**
Focusing	**concentrating on a single point**
Exploring	**delving further into a subject or an idea**
Giving information	**making available the facts that the client needs**
Reflecting	**directing client actions, thoughts, and feelings back to client**
Restating	**repeating the main idea expressed**

Chapter Five

Communication



4. Avoiding Nontherapeutic Communication

Nontherapeutic communication technique	Meaning
Advising	telling the client what to do. Giving advice implies that only the nurse knows what is best for the client
Agreeing	indicating accord with the client
Challenging	demanding proof from the client
Disagreeing	opposing the client's ideas
Indicating the existence of an external source	attributing the source of thoughts, feelings, and behavior to others or to outside influences

Probing	persistent questioning of the client
Reassuring	indicating there is no reason for anxiet or other feelings of discomfort
Testing	appraising the client's degree of insight

Nonverbal communication skills

Nonverbal communication is the behavior that accompanies verbal content such as

1. Body Language

Gestures, postures, movements, and body positions

2. Facial Expression

Facial expressions can be categorized into expressive, impassive, and confusing:

- An expressive face reflects the person's moment-by moment thoughts, feelings, and needs.
- An impassive face is frozen into an emotionless expression similar to a mask.

- A confusing facial expression reflects the opposite of what the person wants to convey

3. Eye Contact

Looking into the other person's eyes during communication.

The nurse must not "stare" at the client.

4. Silence

The client may be depressed and struggling to find the energy to talk.

5. Vocal Cues

Vocal cues are nonverbal sound signals transmitted along with the content: voice volume, tone, pitch, intensity, emphasis, speed, and pauses augment the sender's message.

Verbal Developing Client-practitioner Rapport

Rapport is the ability to relate others in a way that create level of trust and understanding.

Barriers of communication

1. Physiological barriers

*Poor listening skills

* Information overload

* Inattention

* Emotions
* Poor retention
* Physical and environmental distraction

2. Psychological Barriers
3. Social Barriers
4. Cultural Barriers
5. Semantic Barriers
6. Linguistic Barriers
7.Past experience

8. Organizational barriers

* Technological Failure

* Time pressures

* Complexity in organizational structure

8. Organizational barriers

* Technological Failure

* Time pressures

* Complexity in organizational structure

9. Barriers Related with message

* Unclear message

* Stereotypes

* Inappropriate Channel

* Lack of feedback

Chapter Six

Loss and Greif

Understanding Loss

Loss is produced by an event which is perceived to be negative by the individuals involved and results in long-term changes to one's social situations, relationships, or cognitions

Personal losses may include loss of valued social roles and daily routine, loss of one's former self, loss of relationships, and a sense of loss of the future.

Concept of loss

- An actual or potential situation in which a valued object, person, or other aspect is inaccessible or changed so that it is no longer perceived as valuable.
- A condition whereby an individual experiences deprivation of, or complete lack of, something that was previously present.

Types of Loss

- **Suddenly** (eg, death of a child due to an auto accident)
- **Gradually** (eg, loss of a leg due to the progression of peripheral vascular disease).
- **Predictable** or **unexpectedly**
- **Actual** (the loss has occurred or is occurring),

Observable to others or perceived (the loss is recognized only by the client and usually involves an ideal or fantasy), **Anticipatory** (the client is aware that a loss will occur), temporary, or permanent.

Ambiguous loss to describe the situation in which the person is not fully sure if a loss exists is be grieved even when distress is experienced. Ambiguous loss is the incomplete or uncertain loss.

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Understanding Grief

The concept of grief may reflect the reaction to loss

Grief is a normal, appropriate emotional response to an external and consciously recognized loss.

Anticipatory grief

refers to the reactions that occur when an individual, family, significant other, or friends are expecting a loss or death to occur.

Bereavement (e.g., feelings of sadness, insomnia, poor appetite, deprivation, and desolation.

Grief Theory

Grief theory proposes that grief occurs as a process. The grief process is all-consuming, having a physical, social, cultural, spiritual, and psychological impact on an individual that may impair daily functioning.

■ Stages of Grieving and Dying

- **Denial:**

A disbelief in the prognosis of inevitable death. This stage serves as a temporary escape from reality

- **Anger:**

"Why me?" "Why now?" and "It's not fair!" are a few of the comments commonly expressed during this stage.

- **Bargaining:**

Statements such as “If I promise to take my medication, will I get better?” or “If I get better, I’ll never miss church again” are examples of attempts at bargaining to prolong one’s life.

- **Depression:**

They feel helpless watching the depressed client mourn present and future losses.

- **Acceptance:**

Achieved an inner and outer peace due to a personal victory over fear: "I'm ready to die."

Tasks of Mourning

Mourning is a term used to describe an individual's outward expression of grief regarding the loss of a love object or person.

The individual experiences emotional detachment from the object or person, eventually allowing the individual to find other interests and enjoyments.

Chronic sorrow

Long term sadness that accompanies ongoing loss and sometimes sit uncomfortably on the periphery of the consciousness.

■ Grief Reactions

Physical Grief Reactions

Psychological/ Emotional Grief Reactions

Behavioural/ Grief Reactions

Social and Familial Grief Reactions

Spiritual Reactions

■ Grief Reactions

Physical Grief Reactions

Heart palpitation

Trouble breathing

Headaches

Loss /increased appetite

Chills

fatigue

Sleep disturbances
Numbness
Stiff neck /jaw
Nausea/ GI upset
Tremors
Sweating
Muscle weakness

■ Grief Reactions

Psychological/ Emotional Grief Reactions

Irritability/anger

Restlessness

Sadness

Feeling lost

Wanting to be alone

Recurrent dreams
Feeling of frustrtion
Feeling of powerlessness
Hopelesness
Anxiety
Moodiness
Guilt
apathy

■ Grief Reactions

Behavioural/ Grief Reactions

Difficulty in concentration

Forgetfulness

Withdrawing socially

Loss of work efficiency

Hyperactivity

■ Grief Reactions

Social and Familial Grief Reactions

Characteristics such as ascribed roles

Social stigma

Greif guilt

■ Grief Reactions

Spiritual Reactions

The big questions of life can arise

Comfort in faith

Doubts/loss of faith

Anger at God

■ Grief and Post-Traumatic Stress Disorder (PTSD)

Some situations of bereavement loss occur under highly traumatic circumstances – violent crime, road and industrial accidents, suicide, natural disasters, war, torture. In such situations, the effects of the traumatic manner in which the loss occurred complicates the normal grieving.

Chapter Seven

Coping

■ Coping strategies

Problem-focused strategies

Solution-oriented approaches to dealing with a situation that causes stress. If the problem is due to something that can be changed, you can solve it by reducing or eliminating the source of the stress.

Appraisal-focused strategies

Are appropriate when there is no straight-forward solution to a problem. Instead of changing the cause, you modify how you think.

Emotion-focused strategies

involve dealing with the feelings that are stirred up as a consequence of the stressor. This can include managing hostile feelings by counting to ten, or reducing anxiety by meditating or using relaxation techniques., emotion-focused coping is more tactical than strategic, since it is aimed at the effects of the stress.

■ Coping Skills

Self Soothing

- **Breathing:** Take deep breaths from the diaphragm or stomach.
- **Exercise:** Change your focus, get your body moving
- **Music:** Use a relaxing or lively tune or music that inspires you. Consider dancing.

- **Relax your body:** Progressive muscle relaxation.
- **Do yoga. Comfort Item**
- **Imagery:** Look at and direct your attention towards picture or photograph of a beautiful and calming scene (i.e., beach, mountains).
- **Visualization:** Visualize a place you love

Do Something Positive

- Do One Positive Thing.
- Socialize
- Entertain yourself:
- List your achievements:
- Prayer:
- Problem Solving:

Dealing with your thoughts and assumptions

- Self-statements/Cognitive Coping:
- Redirect Unhelpful Thoughts:
- Reality Check:

Distracting

- Focus on your Senses:
- Mindful Eating:
- Mindful Walking:

■ Defense Mechanisms

Examples of Defenses Mechanisms

- **Identification with the Aggressor**

A focus on negative or feared traits.

- **Repression**

Repression is an unconscious mechanism employed by the ego to keep disturbing or threatening thoughts from becoming conscious.

- **Projection**

This involves individuals attributing their own thoughts, feeling and motives to another person. Thoughts most commonly projected onto another are the ones that would cause guilt such as aggressive and sexual fantasies or thoughts.

- **Displacement**

Displacement is the redirection of an impulse (usually aggression) onto a powerless substitute target.

- **Sublimation**

This is similar to displacement, but takes place when we manage to displace our emotions into a constructive rather than destructive activity.

- **Denial**

Denial involves blocking external events from awareness.

- **Regression**

This is a movement back in psychological time when one is faced with stress.

- **Rationalization**

Rationalization is the cognitive distortion of "the facts" to make an event or an impulse less threatening.

- **Reaction Formation**

This is where a person goes beyond denial and behaves in the opposite way to which he or she thinks or feels.

■ Counseling

Several skills need to be brought into a one-to-one counselling session. These include:

1. attitudinal skills;
2. listening skills;
3. verbal communication skills;
4. giving leads.

Chapter Eight

Psychiatric Disorders



■ The definition of Mental Illness

An illness or syndrome with psychological or behavioral manifestation and/or impairment in functioning as a result of a social, psychological, genetic, physical/chemical, or biological disturbance.

The Causes of Mental Illness

Although the exact cause of most mental illnesses is not known, it is becoming clear through research that many of these conditions are caused by a combination of biological, psychological, and environmental factors.

1. Known or unrecognized medical condition.

2. Biological (Genetic factors, infections, brain defect or injury, substance abuse, others)

3. Psychological theories:

4. Environmental factors

Major depressive disorder (MDD)

Diagnostic Criteria

A. *Five (or more)* of the following symptoms have been present during the same *2-week* period *nearly every day* and represent a change from previous functioning: at least one of the symptoms is either (1) depressed mood or (2) loss of interest or pleasure, (all symptoms indicated by either subjective account from individual or observation by others):

1. Depressed mood.

2. Markedly decreased interest or pleasure in nearly all activities.

3. Significant unintentional weight loss or gain, or decrease or increase in appetite.

4. Insomnia or hypersomnia.

5. Psychomotor agitation or retardation.

6. Fatigue or loss of energy.

7. Feelings of worthlessness or excessive or inappropriate guilt.

8. Diminished ability to think or concentrate, or indecisiveness.

9. Recurrent thoughts of death (not just fear of dying), recurrent suicidal ideation without a specific plan, or a suicide attempt or a specific plan for committing suicide.

B. The symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

C. The episode is not attributable to the physiological effects of a substance or to another medical condition or to another psychiatric disorders.

■ Persistent Depressive Disorder (Dysthymia)

A more chronic form of depression, persistent depressive disorder (dysthymia), which characterized by at least 2 years of depressed mood most of the day, for majority of days with two (or more) of additional, less severe symptoms that do not meet the criteria for a major depressive episode. During the 2-year period of the disturbance, the individual has never been without the symptoms for more than 2 months at a time.

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■ Bipolar I Disorder

Diagnostic criteria

For a diagnosis of bipolar I disorder, it is necessary to meet the following criteria for a manic episode. *At least one manic episode* may have been preceded by and may be followed by hypo-manic or major depressive episodes.

- **Manic Episode**

A. A distinct period of abnormally and persistently elevated, expansive, or irritable mood and increased goal-directed activity or energy, lasting at *least 1 week* and present most of the day, nearly every day (or any duration if hospitalization is necessary).

B. Three (or more) of the following symptoms (four if the mood is only irritable) are present to a significant degree and represent a noticeable change from usual behavior:

1. Increased self-esteem or grandiosity.

2. Decreased need for sleep (e.g., feels rested after only 3 hours of sleep).

3. Very talkative or pressure to keep talking.

4. Flight of ideas or thoughts are racing.

5. Distractibility.

6. Increase in goal-directed activity (either socially, at work or school, or sexually) or psychomotor agitation.

7. Excessive involvement in activities that have a high potential for painful consequences (e.g., engaging in unrestrained buying sprees, or foolish business investments).

C. It cause marked impairment in social or occupational functioning or to necessitate hospitalization to prevent harm to self or others, or there are psychotic features.

D. The disturbance is Not due to other psychological condition, or to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition.

- **Hypomanic Episode**

Hypomania is a period of abnormally and persistently elevated, expansive, or irritable mood lasting at least *4 consecutive days* and present most of the day, nearly every day and including three or four of the additional symptoms described earlier in manic episode.

The *difference* is that hypomanic episodes do not impair the person's ability to function (in fact, he or she may be quite productive), and there are no psychotic features.

- **Major Depressive Episode**

Lasts at least *2 weeks*, during which the person experiences a depressed mood or loss of pleasure in nearly all activities.

***Four* of the following symptoms are present every day and cause impairment in functioning:**
Changes in appetite or weight, sleep, or psychomotor activity; decreased energy; feelings of worthlessness or guilt; difficulty thinking, concentrating, or making decisions; or recurrent thoughts of death or suicidal ideation, plans, or attempts.

■ Bipolar II Disorder

At least one episode of major depression and at least one hypomanic episode,

It is no longer thought to be a "milder" condition than bipolar I disorder, largely because of the amount of time individuals with this condition spend in depression and because the instability of mood experienced by individuals with bipolar II disorder is typically accompanied by serious impairment in work and social functioning.

Diagnostic criteria:

A. At least one a current or past hypomanic episode and the criteria for at least one a current or past major depressive episode. There has *never* been a manic episode.

B. It causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

■ Cyclothymic disorder

At least 2 years (for children, a full year) of both hypomanic and depressive periods, and that do not meet criteria for hypomanic episode or a major depressive episode.

The hypomanic and depressive periods have been present for at least half the time and the individual has not been without the symptoms for more than 2 months at a time. The symptoms cause significant distress or impairment in social, occupational, or other areas of functioning.

■ Treatment

Psychopharmacology

Treatment for bipolar disorder involves a lifetime regimen of medications: either an antimanic agent called lithium or anticonvulsant medications used as mood stabilizers (see Chapter 15). This is the only psychiatric disorder in which medications can prevent acute cycles of bipolar behavior.

Psychotherapy

Psychotherapy can be useful in the mildly depressive or normal portion of the bipolar cycle. It is not useful during acute manic stages because the person's attention span is brief and he or she can gain little insight during times of accelerated psychomotor activity. Psychotherapy combined with medication can reduce the risk for suicide and injury, provide support to the client and family, and help the client to accept the diagnosis and treatment plan.

■ Social Anxiety Disorder (Social Phobia)

Diagnostic Criteria

A. Marked persistent fear or anxiety about one or more social situations in which the individual is exposed to possible scrutiny by others.

Examples include social interactions (e.g., having a conversation, meeting unfamiliar people), being observed (e.g., eating or drinking), and performing in front of others (e.g., giving a speech).

B. The individual fears that he or she will act in a way or show anxiety symptoms that will be negatively evaluated (i.e., will be humiliating or embarrassing: will lead to rejection or offend others)

C. The social situations almost always provoke fear or anxiety also avoided or endured with intense fear or anxiety.

■ Panic disorder

Recurrent unexpected panic attacks and is persistently concerned or worried about having more panic attacks or changes his or her behavior in maladaptive ways because of the panic attacks (e.g., avoidance of exercise or of unfamiliar locations).

Panic attacks are abrupt surges of intense fear or intense discomfort that reach a peak within minutes, accompanied by physical and/or cognitive symptoms.

Panic attacks may be expected, or unexpected.

During panic attacks four (or more) of the following symptoms occur:

1. **Palpitations, pounding heart, or accelerated heart rate.**
2. **Sweating.**
3. **Trembling or shaking.**
4. **Sensations of shortness of breath or smothering.**
5. **Feelings of choking.**
6. **Chest pain or discomfort.**

7. Nausea or abdominal distress.

8. Feeling dizzy, unsteady, light-headed, or faint.

9. Chills or heat sensations.

10. *Paresthesias* (numbness or tingling sensations).

11. *Derealization* or *depersonalization.*

12. Fear of losing control or "going crazy."

13. Fear of dying.

■ Treatment

Medications: (Anti-anxiety, Antidepressants, Beta-blockers). Beta-blokers used for heart conditions, are sometimes used to control physical symptoms of anxiety

Psychotherapy

Self-Help, Coping, and Managing

Stress management techniques, deep breathing, relaxation and meditation can be helpful. Support groups provide an opportunity to share experiences and coping strategies.

Learning more about the specifics of a disorder and helping family and friends to understand better can also be helpful. Avoid caffeine, which can worsen symptoms, and check with your doctor about any medications.

***Cognitive behavior therapy (C.B.T)*, a type of talk therapy, can help a person learn a different way of thinking, reacting and behaving to help feel less anxious.**

Behavioral therapy often used to treat phobias include:

***Systematic (serial) desensitization*, (gradually exposes the client to the threatening object in a safe setting until the client's anxiety decreases).**

***Flooding* (confronts the client with the phobic object) until it no longer produces anxiety.**

■ Posttraumatic Stress Disorder

Diagnostic Criteria

A. ***Exposure*** to actual or threatened death, serious injury, or sexual violence in one (or more) of the following ways:

- Directly experiencing the traumatic event.
- Witnessing, in person, the event as it occurred to others.

- **Learning that the traumatic event occurred to a close family member or close friend. In cases of actual or threatened death of a family member or friend, the event must have been violent or accidental.**
- **Experiencing repeated or extreme exposure to aversive details of the traumatic event (e.g., police officers repeatedly exposed to details of child abuse)**

B. Presence of one (or more) of the following *intrusion symptoms* associated with the traumatic event, beginning after the traumatic event occurred:

- **Recurrent, involuntary, and intrusive distressing memories of the traumatic event.**
- **Recurrent distressing dreams that related to the traumatic event.**
- **Dissociative reactions (e.g., flashbacks) in which the individual feels or acts as if the traumatic event were recurring.**

- **Intense or prolonged psychological distress at exposure to internal or external cues that symbolize or resemble an aspect of the traumatic event.**
- **Marked physiological reactions to internal or external cues that symbolize or resemble an aspect of the traumatic event.**

C. ***Persistent avoidance*** or efforts to avoid stimuli associated with the traumatic event, beginning after the traumatic event occurred, as evidenced by ***one or both*** of the following:

- Avoidance of distressing memories, thoughts, or feelings about the traumatic event.
- Avoidance of external reminders (people, places, conversations, activities, objects, situations) that arouse distressing memories, thoughts, or feelings about or closely associated with the traumatic event.

D. *Negative alterations in cognitions and mood* **associated with the traumatic event, beginning or worsening after the traumatic event occurred, as evidenced by two (or more) of the following:**

- **Inability to remember an important aspect of the traumatic event (typically due to dissociative amnesia only).**
- **Persistent and exaggerated negative beliefs or expectations about oneself, others, or the world (e.g., "No one can be trusted," 'The world is completely dangerous,").**

- Persistent, distorted cognitions about the cause or consequences of the traumatic event that lead the individual to blame himself/herself or others.
- Persistent negative emotional state (e.g., fear, horror, anger, guilt, or shame).
- Markedly diminished interest or participation in significant activities.
- Feelings of detachment or estrangement from others.
- Persistent inability to experience positive emotions.

E. Marked *alterations in arousal and reactivity* associated with the traumatic event, beginning or worsening after the traumatic event occurred, as evidenced by two (or more) of the following:

- **Irritable behavior and angry outbursts (with little or no provocation) typically expressed as verbal or physical aggression toward people or objects.**
- **Reckless or self-destructive behavior.**

- **Hypervigilance.**
- **Exaggerated startle response.**
- **Problems with concentration.**
- **Sleep disturbance (e.g., difficulty falling or staying asleep or restless sleep).**

F. Duration of the disturbance (Criteria B, C, D, and E) is *more than 1 month.*

G. The disturbance causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

H. The disturbance is not attributable to the physiological effects of a substance (e.g., medication, alcohol) or another medical condition. For summary of symptoms, see Table 8.1

Specify whether:

- **With dissociative symptoms:** The individual's symptoms meet the criteria for posttraumatic stress disorder, and in addition, in response to the stressor, the individual experiences persistent or recurrent symptoms of either of the following:

1. ***Depersonalization***

■ Treatment

PTSD treatment can help the client to regain a sense of control over their life. The primary treatment is psychotherapy, but often included medication. Combining these treatments is necessary.

Psychopharmacology

Several types of medications can help in the improvement of symptoms of PTSD; which include antidepressants, antianxiety, or hypnotics medications.

Psychotherapy (Talk therapy)

- ***Cognitive therapy*** **helps the client to recognize the ways of thinking that are keeping him/her stuck, its often used along with exposure therapy.**

- ***Exposure therapy*** **helps the client to face frightening situation and cope with it effectively.**

- ***Eye movement desensitization and reprocessing*** **(EMDR) combining exposure therapy with a serious of guided eye movements that help to process traumatic memories and change the way of reacting to it.**

■ Conversion Disorder (Functional Neurological Symptom Disorder)

Conversion disorder is a loss of or change in body function that cannot be explained by any known medical disorder or pathophysiological mechanism. There is most likely a psychological component involved in the initiation, exacerbation, or perpetuation of the symptom, although it may or may not be obvious or identifiable.

Conversion symptoms affect voluntary motor or sensory functioning suggestive of neurological disease.

Examples include:

Paralysis,
Aphonia **(inability to produce voice),**
Seizures,
Coordination disturbance,
Difficulty swallowing,
Urinary retention,
Akinesia.

Blindness,
Deafness,
Double vision,
***Anosmia* (inability to perceive smell),**
Loss of pain sensation,
Hallucinations
***Pseudocyesis* (false pregnancy) may represent a strong desire to be pregnant.**

It is likely that multiple causes play a role in the etiology. Most symptoms of

■ Treatment

Treatment focuses on managing symptoms and improving quality of life. The health care provider must show empathy and sensitivity to the client's physical complaints. A trusting relationship helps to ensure that clients stay with and receive care from one provider instead of "doctor shopping." Optimal treatment combines medication and psychotherapy.

For many clients, depression may accompany or result from somatoform disorders. Thus, antidepressants help in some cases. Selective serotonin reuptake inhibitors are used most commonly.

For clients with pain disorder, referral to a chronic pain clinic may be useful. Clients learn methods of pain management such as visual imaging and relaxation.

Services such as physical therapy to maintain and build muscle tone help to improve functional abilities. Providers should avoid prescribing and administering narcotic analgesics to these clients because of the risk for dependence or abuse. Clients can use nonsteroidal anti-inflammatory agents to help reduce pain.

Involvement in a structured cognitive-behavioral group is beneficial for some people with somatoform disorders, especially somatization disorders.

CBT help in improvement of physical and emotional health, and may alter the erroneous perceptions of threat (of illness) in hypochondriasis.

The overall goals of the group were offering peer support, sharing methods of coping, and perceiving and

Chapter Nine

Abuse and Neglect

■ What is abuse?

Abuse is defined in Devon's and Torbay's policy and guidance as follows: "Abuse is a violation of an individual's human and civil rights by another person or persons."

There are different forms of abuse, as described below.

1.1 Psychological Abuse

This may include:

- Emotional abuse.
- Threats of harm or abandonment.
- Deprivation of contact.
- Humiliation.
- Blaming.

Signs that psychological abuse may be taking place could include:

- Low self-esteem.
- Lack of confidence and anxiety.
- Increased levels of confusion.
- Increased urinary or faecal incontinence.
- Sleep disturbance.

1.2 Physical Abuse

Physical abuse may include:

- Hitting.
- Slapping.
- Pushing.
- Kicking.
- Misuse of medication.
- Restraint or inappropriate sanctions.

Signs that physical abuse may be taking place can be:

- Injuries that are consistent with physical abuse.
- Injuries that are the shape of objects.
- Presence of several injuries of a variety of ages.
- Injuries that have not received medical attention.

1.3 Sexual Abuse

Sexual abuse may include:

- Rape and sexual assault to which the vulnerable adult has not consented, could not consent, or was pressurised into consenting.
- Non-contact sexual abuse could include being forced or coerced to be photographed or videoed to allow others to look at their body.

Signs that sexual abuse may be taking place:

- Sexually transmitted diseases or pregnancy.
- Tears or bruises in genital/anal areas.
- Soreness when sitting.
- Signs that someone is trying to take control of their body image, for example, anorexia, bulimia or self-harm.

•1.4 Neglect

Neglect can include:

- Ignoring medical or physical care needs.
- Failure to provide access to appropriate health, social care or educational services.
- The withholding of the necessities of life, such as medication, adequate nutrition and heating.

Signs that neglect may be occurring:

- Malnutrition.
- Rapid or continuous weight loss.
- Not having access to necessary physical aides.
- Inadequate or inappropriate clothing.
- Untreated medical problems e.g. pressure ulcers
- Dirty clothing/bedding.
- Lack of personal care.

■ Abuse of Individuals with Disabilities

four factors that increase the risk of abuse.

1. The disability itself
2. Myths and stereotypes about persons with disabilities
3. Dependency
4. Isolation

Chapter Ten

Self-Destructive Behaviors

■ Introduction

Self-destructive behaviors are any behaviors that negatively impact our mind or body by the life choices we make. Most people are unaware of their self-destructive habits. Usually there is some problem with handling or expressing feelings.

■ Anorexia Nervosa

Symptomatology

1. Morbid fear of obesity
2. Refusal to eat.
3. Preoccupation with food.
5. Delayed psychosexual development.
6. Compulsive behavior
7. Extensive exercising is common.
8. Feelings of depression and anxiety ofter accompany this disorder.
9. May engage in the binge-and-purge

■ Bulimia Nervosa

"the binge-and-purge syndrome"

Diagnostic criteria:

A. Recurrent episodes of ***binge eating***. An episode of binge eating is characterized by both of the following:

- Eating, in a discrete period of time (e.g., within any 2-hour period), an amount of food that is definitely larger than what most individuals would eat in a similar period of time under similar circumstances.

- A sense of lack of control over eating during the episode (e.g., a feeling that one cannot stop eating or control what or how much one is eating).
- Recurrent inappropriate compensatory behaviors in order to prevent weight gain, such as self-induced vomiting; misuse of laxatives, diuretics, or other medications; fasting; or excessive exercise.

C. both occur, on average, at least once a week for ***3 months***.

Treatment

Medical management focuses on weight restoration, nutritional rehabilitation, rehydration, and correction of electrolyte imbalances.

Antidepressants and the **antihistamine**

Family therapy

Individual therapy

Cognitive behavioral therapy has been found to be the most effective treatment for bulimia.

■ Self-injuries behavior

Self-injury typically refers to a variety of behaviors in which an individual intentionally inflict harm to his or her body for purposes not socially recognized or sanctioned and without suicidal intent.

They often share the following characteristics: